PART A: QUESTIONS 1 TO 12

In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient.

For **questions 1 to 24**, complete the notes with information you hear in the recording.

Extract 1: Questions 1 to 12

You hear a foundation doctor talking to a recently admitted patient called Roy Miller. For **questions 1 to 12**, complete the notes with a word or short phrase.

Name	Roy Miller
Reasons for admission	• shortness of breath — difficulty walking **(1)**_____, often becomes short of breath — breathlessness has increased • coughing and wheezing — sounds like '**(2)**_____' and hasn't improved — worse when **(3)**_____, preventing sleep — coughing up phlegm, described as **(4)**_____ in colour

—	phlegm has gradually darkened over the week • suffering hot and cold spells, feels **(5)**_____
Medical history	• diagnosed with **(6)**_____ last year • was a **(7)**_____, stopped six years ago • occasionally suffers from gout (treated with **(8)**_____) • arthritis located in **(9)**_____
Medication	• using **(10)**_____ more frequently • takes a statin for **(11)**_____ • occasionally uses paracetamol for arthritis • **(12)**_____ causes an allergic reaction

QUESTIONS 13 TO 24

Extract 2: Questions 13 to 24

You hear an optometrist talking to a patient called Marsha Samarina. For **questions 13 to 24**, complete the notes with a word or short phrase.

Patient	Marsha Samarina
Description of initial symptoms	• pain in eye, felt like something was **(13)**_____ • noticed headache • eye was '**(14)**_____' profusely (compares to chopping

	onions) • roommate noticed that eye was **(15)**_____, took to ER • pain was severe, unable to **(16)**_____
Initial GP treatment	• unable to identify cause, performed **(17)**_____ test • GP suggested possibility of **(18)**_____
Optometrist treatment	• given **(19)**_____ for pain-relief • eye exam showed **(20)**_____
At home treatment	• advised to avoid wearing contacts • prescribed: — **(21)**_____ - using twice daily — and **(22)**_____ less often, finds application - unpleasant • also taking **(23)**_____ for pain relief
Current condition	• condition has improved • pain caused by **(24)**_____

PART B: QUESTIONS 25 TO 30

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25 to 30**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear.

25. You hear two doctors discuss the transfer of care for a patient.

The patient’s CURB-65 score means that he will

(A) be transferred from the Emergency Department.
(B) receive additional medication and treatment.
(C) be treated as an out-patient.

26. You hear a speech pathologist talking to the wife of a patient who has recently suffered a stroke.

What does she want to know about her husband’s condition?

(A) how long it will take him to make a full recovery

(B) whether his communication issues will improve

(C) what she can do to speed the healing process

27. You hear a trainee doctor asking a senior colleague about chest tubes.

What is the senior colleague doing?

(A) explaining how to use them correctly
(B) recommending an alternative to them
(C) demonstrating what can go wrong with them

28. You hear a pharmacist talking to a customer about pain relief.

What has the customer been misinformed about?

(A) the stock of medication in the pharmacy
(B) the usefulness of a type of pain relief
(C) the availability of a medicine

29. You hear a trainee nurse receiving feedback from his tutor

What does she explain?

(A) listening to a patient’s concerns is essential

(B) how to become more self-assured when interacting with patients

(C) the importance of providing adequate emotional support to patients

30. You hear two doctors planning their patient-care schedule

What is their priority?

(A) identifying the patients at greatest risk
(B) dealing with patients who need tests arranging
(C) ensuring that all patients have key documentation

PART C: QUESTIONS 31 TO 36

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31 to 42**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear.

Extract 1: Questions 31 to 36

You hear an interview with Dr Matthew Leach, who's talking about meningitis caused by *Neisseria meningitides*.

31. Dr Leach says that during the onset of meningitis, many patients

(A) do not realise they are unwell.
(B) mistake the illness for something else.
(C) experience life-threatening symptoms.

32. Dr Leach says that meningitis is common in college students because of their

(A) poor hygiene habits.
(B) proximity to new people.
(C) weakened immune systems.

33. Why does Dr Leach say the patient didn’t seek treatment sooner?

(A) He was unsure of what to do.
(B) He didn’t think he needed treatment.
(C) He was trying to finish his assignments.

34. Dr Leach began treating for meningitis before receiving the spinal fluid results because

(A) the illness progresses rapidly.
(B) the treatment is the same for all causes.
(C) the test results did not affect the diagnosis.

35. Dr Leach explains that meningitis is more likely to cause long term after-effects if

(A) it is not accurately diagnosed.
(B) patients do not seek treatment quickly.
(C) reactions to the virus are extremely severe.

36. Dr Leach advises those who think they may be infected with meningitis to

(A) get vaccinated at the earliest opportunity.
(B) avoid people who may be suffering from the virus.
(C) take precautions to prevent others from becoming ill.

QUESTIONS 37 TO 42

Extract 2: Questions 37 to 42

You hear a presentation given by a clinical psychiatrist called Dr Evalina Houghton about agitated patients in an emergency setting.

37. Dr Houghton says that patients in the ED are more likely to be agitated as they are likely to

(A) suffer from untreated health problems.
(B) have been given bad news recently.
(C) require medical help frequently.

38. Dr Houghton explains that creating space between the patient and the provider

(A) enables both parties to remain calm.
(B) encourages the patient to exit the room.
(C) reduces the likelihood of the provider being injured.

39. Why does Dr Houghton encourage providers to speak slowly?

(A) to ensure the patient understands what is being said
(B) to give other members of staff time to prepare
(C) to give the patient an opportunity to speak

40. What approach does Dr Houghton suggest for patients suffering from delusions?

(A) agree with the patient completely
(B) acknowledge the patient’s emotions
(C) explain why their delusions are false

41. Dr Houghton suggests that choices given to the patient should

(A) avoid upsetting the patient by remaining positive.
(B) maintain the patient’s trust by being realisable.
(C) be limited in order to prevent confusion.

42. Dr Houghton recommends that when the patient is calm they should

(A) be removed from the ED ward.
(B) understand why their behaviour was inappropriate.
(C) be encouraged to explain what caused their reaction.

OCCUPATIONAL ENGLISH TEST

Sample Test 1

LISTENING SUB-TEST – QUESTION PAPER

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

Your details and photo will be printed here.

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: APPROXIMATELY **40 MINUTES**

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper.**

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part A: Write your answers on this **Question Paper** by filling in the blanks.

Example: Patient: Ray Sands

Part B & Part C: Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

(A)

(B)

(C)

SAMPLE

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BLANK

SAMPLE

Occupational English Test

Listening Test

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: --beep--

You'll have time to read the questions before you hear each extract and you'll hear each extract **ONCE ONLY.** Complete your answers as you listen.

At the end of the test you'll have two minutes to check your answers.

Part A

In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient.

For **questions 1-24**, complete the notes with information you hear.

Now, look at the notes for extract one.

SAMPLE

SAMPLE

Extract 1: Questions 1-12

You hear a physiotherapist talking to a new patient called Ray Sands. For **questions 1-12**, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes.

Patient Ray Sands

18 months ago
- back injury sustained (lifting **(1)**_____)

1 year ago
- sciatica developed

6 months ago
- clear of symptoms

Last month
- recurrence of symptoms

Patient's description of symptoms

- pain located in **(2)**_____
- pain described as **(3)**_____
- loss of mobility
- problems sleeping
- mentions inability to **(4)**_____ as most frustrating aspect
- **(5)**_____ sensation (calves)
- general numbness in affected area

Occupation
- **(6)**_____ (involves travel/some manual work)

Initial treatment
- prescribed NSAIDs
- application of **(7)**_____ (provided some relief)

Referrals
- **(8)**_____ (briefly)
- sports injury specialist for manipulation and exercise programme

Further treatment
- epidural injections
- **(9)**_____
- electrical impulses
- decided not to try **(10)**_____
- patient attributes recovery to **(11)**_____

Previous diagnosis
- sciatica probably related to **(12)**_____
- reports no history of pain in buttocks

SAMPLE

Extract 2: Questions 13-24

You hear a consultant dermatologist talking to a patient called Jake Ventor. For **questions 13-24**, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes.

Patient Jake Ventor

Reason for referral
- skin lesion

Patient's description of condition
- on the **(13)**_____ of his left hand
- preceded by **(14)**_____
- then **(15)**_____ form and join up
- surrounding erythema
- GP describes appearance of lesion as **(16)**_____
- normally resolves within two weeks

History of condition
- first experienced in 1990s when living in China
- also had a lesion on his **(17)**_____ – never recurred there
- recurs regularly on different parts of his left hand
- not becoming more **(18)**_____
- no apparent link to general state of health, **(19)**_____ or stress

Medical history
- **(20)**_____ on lower back in 2006 – no sign of recurrence
- reports no history of **(21)**_____

Information given
- advised that **(22)**_____ was unlikely to be effective
- told him to take care if the skin is **(23)**_____

Outcome
- says his quality of life isn't affected
- a **(24)**_____ will be arranged

That is the end of Part A. Now look at Part B.

SAMPLE

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A, B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

---

25. You hear a nurse briefing her colleague about a patient.

What does she warn her colleague about?

- (A) The patient is allergic to some types of antibiotics.
- (B) Care must to be taken to prevent the patient from falling.
- (C) Oxygen may be needed if the patient becomes breathless.

26. You hear the manager of a care home for the elderly talking to the nursing staff.

He says that errors in dispensing medication to patients usually result from

- (A) interruptions while calculating dosages.
- (B) a failure to check for patients' allergies.
- (C) administering drugs late in the day.

27. You hear part of a morning briefing on a hospital ward.

What is the plan for the patient today?

- (A) Her emotional state will be carefully observed.
- (B) She will be transferred to a more specialised unit.
- (C) A social worker will come to see what help she needs.

SAMPLE

28. You hear part of an ante-natal consultation at a GP practice.

What does the patient want to know about?

(A) the advisability of a home birth

(B) ways of avoiding post-natal depression

(C) what painkillers might be available during labour

29. You hear a trainee doctor telling his supervisor about a problem he had carrying out a procedure.

The trainee feels the cause of the problem was

(A) treatment administered previously.

(B) the patient's negative reaction.

(C) inappropriate equipment.

30. You hear a doctor talking to a teenage boy who has a painful wrist.

The doctor wants to establish whether

(A) a fracture may be misaligned.

(B) the swelling may be due to a sprain.

(C) there may be more than one bone affected.

That is the end of Part B. Now look at Part C.

SAMPLE

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear an interview with a cardiologist called Dr Jack Robson, who's an expert on Chagas disease.

You now have 90 seconds to read **questions 31-36**.

31. Why does Dr Robson regard Chagas as a neglected disease?

- (A) because of the social groups it mainly affects
- (B) because patients often don't realise they're infected
- (C) because its impact is severe in a relatively small number of cases

32. Dr Robson says that concerns over Chagas in the USA are the result of

- (A) a rise in the number of people at risk of being infected with the disease.
- (B) a greater awareness of how many people there have the disease.
- (C) an increased prevalence of the insect which carries the disease.

33. A patient called Marisol recently asked Dr Robson to test her for Chagas because

- (A) she was worried about the health of any children she might give birth to.
- (B) she wanted to know whether it was safe for her to donate blood.
- (C) she thought she had symptoms associated with the disease.

SAMPLE

34. What problem does Dr Robson identify in the case of a patient called Jennifer?

(A) an unwillingness to accept that she was ill

(B) an inability to tolerate the prescribed medicine

(C) a delay between the initial infection and treatment

35. What does Dr Robson say about his patient called Juan?

(A) The development of his illness was typical of people with Chagas.

(B) An incorrect initial diagnosis resulted in his condition worsening.

(C) The medication he took was largely ineffective.

36. Dr Robson thinks the short-term priority in the fight against Chagas is to

(A) increase efforts to eliminate the insects which carry the parasite.

(B) produce medication in a form that is suitable for children.

(C) design and manufacture a viable vaccine.

Now look at extract two.

SAMPLE

Extract 2: Questions 37-42

You hear an occupational therapist called Anna Matthews giving a presentation to a group of trainee doctors.

You now have 90 seconds to read **questions 37-42**.

37. Anna says that the main focus of her work as an occupational therapist is

- (A) designing activities to meet the changing needs of each patient.
- (B) making sure she supports patients in reaching their goals.
- (C) being flexible enough to deal with patients of all ages.

38. When Anna first met the patient called Ted, she was

- (A) unable to identify completely with his attitude.
- (B) optimistic that he would regain full mobility.
- (C) mainly concerned about his state of mind.

39. Because Ted seemed uninterested in treatment, Anna initially decided to focus on

- (A) what he could achieve most easily.
- (B) allowing him to try and help himself.
- (C) making him come to terms with his injuries.

40. Anna feels that, in the long term, her therapy helped Ted because

- (A) it led him to become less emotional.
- (B) it made him appreciate the need for patience.
- (C) it showed him there was something to work towards.

SAMPLE

41. Anna describes the day Ted had his plaster casts removed in order to

(A) demonstrate how slow any progress can seem to patients.

(B) illustrate the problems caused by raising a patient's hopes.

(C) give advice on what to do when patients experience setbacks.

42. Anna suggests that when patients like Ted recover enough to go home, they are often

(A) too ambitious in what they try to achieve initially.

(B) able to build on the work of the occupational therapist.

(C) held back by the over-protective attitude of family members.

That is the end of Part C.

You now have two minutes to check your answers.

THAT IS THE END OF THE LISTENING TEST

SAMPLE

SAMPLE

BLANK

SAMPLE

OCCUPATIONAL ENGLISH TEST

Sample Test 2

LISTENING SUB-TEST – QUESTION PAPER

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

Your details and photo will be printed here.

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: APPROXIMATELY **40 MINUTES**

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper.**

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part A: Write your answers on this **Question Paper** by filling in the blanks.

Example: Patient: Ray Sands

Part B & Part C: Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

(A)

(B)

(C)

SAMPLE

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SAMPLE

Occupational English Test

Listening Test

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: --beep--

You'll have time to read the questions before you hear each extract and you'll hear each extract **ONCE ONLY.** Complete your answers as you listen.

At the end of the test you'll have two minutes to check your answers.

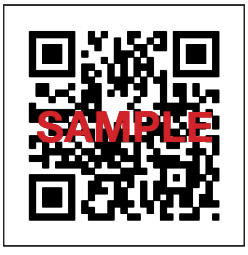
Part A

In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient.

For **questions 1-24**, complete the notes with information you hear.

Now, look at the notes for extract one.

SAMPLE



Extract 1: Questions 1-12

You hear a gastroenterologist talking to a patient called Andrew Taylor. For **questions 1-12**, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes.

Patient Andrew Taylor

Background

- has had **(1)**_____ over long period
- reports a frequent **(2)**_____ sensation in the last year
- most recently **(3)**_____ has become a problem
- word used to describe symptoms – **(4)**_____
- pre-existing skin condition aggravated
- frequent **(5)**_____ – patient didn't initially link these to bowel condition

Effects of condition on everyday life

- works as an **(6)**_____
- situation at work means patient is **(7)**_____
- complains of lack of **(8)**_____
- has noticed an increase in insomnia

Diet

- claims to be consuming sufficient **(9)**_____
- claims to keep hydrated
- has experimented with excluding **(10)**_____ from diet
- very slight reduction in caffeine intake
- has undergone **(11)**_____ – no indications of anything problematic

Medication

- has taken an anti-spasmodic – not very effective
- now trying **(12)**_____

SAMPLE



Extract 2: Questions 13-24

You hear a hospital neurologist talking to a new patient called Kathy Tanner. For **questions 13-24**, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes.

Patient Kathy Tanner

Background to condition

- experienced discomfort and a **(13)**_____ feeling in neck whilst driving
- osteopathy exacerbated problem
- used **(14)**_____ to relieve symptoms in neck

Further developments in condition and diagnosis

- describes a pulling sensation (dragging her head to the right)
- doctor recommended **(15)**_____
- diagnosis of spasmodic torticollis (ST)
  - condition described as **(16)**_____
  - resulted in feelings of depression

Treatment history

(a) from home

- some months of **(17)**_____
- visited two neurologists without success
- prescribed **(18)**_____ (antispasmodic)
- joined an ST support group
- bought **(19)**_____ to provide extra support

(b) from university hospital

- treatment using **(20)**_____ injections
  - side effects included difficulties **(21)**_____
  - reports treatment as increasingly ineffective
- supplemented by **(22)**_____
- experienced confusion and **(23)**_____
- analgesic relief: morphine self-administered via **(24)**_____

That is the end of Part A. Now look at Part B.

SAMPLE

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A, B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

---

25. You hear an optometrist talking to a patient who's trying contact lenses for the first time.

What is the patient concerned about?

- (A) his blurred vision
- (B) soreness in his eyes
- (C) how to remove the lenses

26. You hear a nurse asking a colleague for help with a patient.

Why does the nurse need help?

- (A) The patient's condition has deteriorated.
- (B) The patient is worried about a procedure.
- (C) The patient is reporting increased pain levels.

27. You hear a senior nurse talking about a new initiative that has been introduced on her ward.

What problem was it intended to solve?

- (A) patients' confusion over information given by the doctor
- (B) relatives not being able to discuss issues with the doctor
- (C) patients not discussing all their concerns when meeting the doctor

SAMPLE

28. You hear two radiologists talking about the type of scan to be given to a patient.

They agree to choose the method which will

(A) allow them to see the whole of the appendix.

(B) probably give the most accurate results.

(C) have the fewest risks for the patient.

29. You hear part of a surgical team's briefing.

The male surgeon suggests that the patient could

(A) require specialist equipment during surgery.

(B) benefit from a specific anaesthetic procedure.

(C) be at risk of complications from another health issue.

30. You hear a senior research associate talking about a proposal to introduce inter-professional, primary healthcare teams.

What hasn't been established about the teams yet?

(A) the best way for collaboration to take place

(B) the financial impact that they are likely to have

(C) the aspects of medical care they are best suited to

That is the end of Part B. Now look at Part C.

SAMPLE

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear a presentation by a specialist cancer nurse called Sandra Morton, who's talking about her work with prostate cancer patients, including a man called Harry.

You now have 90 seconds to read **questions 31-36**.

31. What does Sandra Morton see as the main aim in her work?

(A) to inform patients about the different treatments on offer

(B) to publicise the availability of tests for the condition

(C) to raise awareness of the symptoms of the illness

32. When Harry was offered a routine health check at his local surgery, he initially

(A) resisted the idea due to his wife's experience.

(B) felt that he was too fit and well to be in need of it.

(C) only agreed to attend because his doctor advised him to.

33. During Harry's investigations for prostate cancer at a hospital clinic, he

(A) felt part of the examination procedure was unpleasant.

(B) found it hard to cope with the wait for some results.

(C) was given false hope by a preliminary blood test.

SAMPLE

34. What was Harry's response to being diagnosed with prostate cancer?

(A) He found himself reacting in a way he hadn't anticipated.

(B) He was unconvinced by the prognosis he was given.

(C) He immediately started researching treatment options online.

35. What typical patient response to the illness does Sandra mention?

(A) an unwillingness to commence appropriate medication

(B) a failure to seek advice regarding different treatment options

(C) a reluctance to talk about the embarrassing aspects of treatment

36. Sandra believes that community follow-up clinics are important because they

(A) offer patients more personal aftercare.

(B) are proven to be less traumatic for patients.

(C) provide rapid treatment for patients developing new symptoms.

Now look at extract two.

SAMPLE

Extract 2: Questions 37-42

You hear a neurologist called Dr Frank Madison giving a presentation about the overuse of painkillers.

You now have 90 seconds to read **questions 37-42**.

37. In Dr Madison's experience, patients who become addicted to painkillers

- (A) are more likely to move on to hard drugs.
- (B) come from a wide variety of backgrounds.
- (C) usually have existing psychological problems.

38. Dr Madison thinks some GPs over-prescribe opioid painkillers because these

- (A) have a long-standing record of success.
- (B) enable them to deal with patients more quickly.
- (C) represent a relatively inexpensive form of treatment.

39. Dr Madison regrets that management of acute pain

- (A) is often misunderstood by the general public.
- (B) receives inadequate attention in medical training.
- (C) fails to distinguish between different possible triggers.

40. Dr Madison's main concern about painkillers being readily available is that

- (A) patients may build up a resistance to them.
- (B) they may be taken in dangerous amounts by patients.
- (C) they may interact adversely with patients' other medication.

SAMPLE

41. Dr Madison refers to the case of an osteoarthritic patient called Ann to highlight

(A) the unsuitability of opioids for patients with particular conditions.

(B) the effect on patients' working lives of dependence on painkillers.

(C) the extreme fear patients may have of living without pain medication.

42. Ann's GP initially failed to identify her dependence because

(A) she managed to conceal its physical effects from him.

(B) he was unaware that she had another source of drugs.

(C) he lacked experience in dealing with problems like hers.

That is the end of Part C.

You now have two minutes to check your answers.

THAT IS THE END OF THE LISTENING TEST

SAMPLE

BLANK

SAMPLE

OCCUPATIONAL ENGLISH TEST

Mock test

LISTENING SUB-TEST – QUESTION PAPER

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:
CANDIDATE SIGNATURE _____

Passport Photo

TIME: APPROXIMATELY **40 MINUTES**

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper**.

DO NOT remove the OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part A: Write your answers on this Question Paper by filling in the blanks.

Example: Patient: Ray Sands

Part B & Part C: mark your answers on the **Question Paper** by filling in the circle using a 2B pencil.

Example:
Ⓐ
Ⓑ
Ⓒ

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BLANK

Occupational English Test

Listening Test

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: --beep—

You'll have time to read the questions before you hear each extract and you'll hear each extract **ONCE ONLY**. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A

In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient.

For **questions 1-24**, complete the notes with information you hear.

Now, look at the notes for extract one.

Extract 1: Questions 1-12

You hear a consultant rheumatologist talking to a patient called Suzanne Hinds. For **questions 1-12**, complete the notes with a word or short phrase.

You now have 30 seconds to look at the notes.

Patient Suzanne Hinds

Symptoms

Mouth:

- chapped lips
- painful **(1)** _____
- increased number of **(2)** _____
- dryness
- tongue appears **(3)** _____

Eyes:

- eyelids described as **(4)** _____ on waking
- frequent irritation
- itchiness made worse by **(5)** _____
- self-treating with **(6)** _____
- recurring **(7)** _____
- increased sensitivity to light
- vision described as cloudy

Throat:

- patient says it sometimes feels **(8)** _____
- regular difficulty in **(9)** _____
- some swelling

Background details

- works as a tour guide
- moderate smoker for 15 years
- family history of **(10)** _____

Recommended tests

- saliva flow rate test
- **(11)** _____biopsy
- **(12)** _____test - possibly

Extract 2: Questions 13-24

You hear a gastroenterologist talking to a patient called Toby Smithers. For **questions 13-24**, complete the notes with a word or short phrase.
You now have thirty seconds to look at the notes.

Patient Toby Smithers

Background

- initial stomach upset
- main symptom **(13)**_____

Original presenting factors

- feeling **(14)** _____ immediately after meals
- extremely **(15)** _____ (especially at night)
- no history of **(16)** _____

GP's initial diagnosis

- main symptoms similar to those associated with **(17)** _____

GP's interim treatment plan

- advised to keep **(18)** _____ intake high
- suggested separating eating and drinking by 30 mins
- suggested a **(19)** '_____' regime for meals

Tests

- both **(20)** _____ clear
- **(21)** _____ performed
- **(22)** _____confirmed and treated
- eradication confirmed by **(23)** _____

Current situation

- original presenting factor persists
- works as a **(24)**_____

That is the end of Part A. Now look at Part B.

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A, B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

---

25. You hear a consultant talking to a woman whose father has just been admitted to hospital.

What does she want to know about his condition?

Ⓐ how serious it is

Ⓑ how much pain it's causing

Ⓒ how long before there will be a diagnosis

26. You hear a trainee nurse receiving feedback from her tutor about the ward round she's just completed.

What would have improved the nurse's performance?

Ⓐ eliciting information from the patient

Ⓑ keeping the patient better informed

Ⓒ updating patient notes more fully

27. You hear a hospital nurse briefing a colleague about a patient with Chronic Obstructive Pulmonary Disease, or COPD.
What does he want his colleague to do?

Ⓐ encourage greater mobility

Ⓑ organise a visit by a dietitian

Ⓒ consult with the medical team

28. You hear two hospital managers talking about a training session for people who do voluntary work with patients.

What do the managers think about the course?

Ⓐ Too few people attended to make it worthwhile.

Ⓑ The content may need revising for future sessions.

Ⓒ The feedback from participants wasn't encouraging.

29. You hear a pharmacist talking to a doctor about a patient's medication.

What is the pharmacist doing?

Ⓐ reporting side effects

Ⓑ checking the dosage

Ⓒ recommending an alternative

30. You hear a surgeon talking to a member of his team as they are finishing a surgical procedure.

The surgeon says their next priority should be

Ⓐ to complete a routine administrative task.

Ⓑ to report a faulty piece of equipment.

Ⓒ to locate an appropriate bed.

That is the end of Part B. Now look at Part C.

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear an interview with Dr Bob Dean, who's talking about a trial he conducted to assess different ways of treating the condition known as 'tennis elbow'.

You now have 90 seconds to read **questions 31-36**.

31. Dr Dean says that patients with tennis elbow

- Ⓐ may be unaware that they have the condition at first.
- Ⓑ tend to come from a remarkably narrow range of occupations.
- Ⓒ can easily avoid the condition by adopting correct working practices.

32. In the physiotherapy programme used in Dr Dean's trial,

- Ⓐ the treatment given was tailored to the needs of each patient.
- Ⓑ patients had to build up their strength before starting it.
- Ⓒ some patients found the treatment too painful.

33. What does Dr Dean say about the role of 'smart rest' in the trial?

- Ⓐ It wasn't appropriate for certain types of tennis elbow.
- Ⓑ It formed the basis of two of the three treatment options.
- Ⓒ It kept all patients physically active despite the condition.

34. What did Dr Dean find interesting about the results of the trial?

Ⓐ They were surprisingly conclusive in the short term.

Ⓑ They underlined the advantages of timely intervention.

Ⓒ They confirmed the findings of an earlier piece of research.

35. Dr Dean would advise anyone experiencing tennis elbow for more than three months to

Ⓐ avoid using any steroid-based medication at that point.

Ⓑ be ready to start more invasive forms of treatment.

Ⓒ try a combination of physiotherapy and injections.

36. Dr Dean suggests that taking anti-inflammatories for tennis elbow

Ⓐ is less effective than alternative forms of medication.

Ⓑ is trying to treat a symptom that may not actually exist.

Ⓒ is an area that needs to be researched more thoroughly.

Now look at extract two.

Extract 2: Questions 37-42

You hear a presentation in which a researcher called Dr Sarah Jones is talking on the subject of weight loss interventions by GPs.

You now have 90 seconds to read **questions 37-42**.

37. Dr Jones suggests that few health professionals currently attempt weight loss interventions because they

- Ⓐ have often found them to be ineffective.
- Ⓑ lack confidence in the chances of success.
- Ⓒ rarely have time to spare for non-clinical issues.

38. The specific aim of the trial Dr Jones describes was

- Ⓐ to draw patients' attention to the need to lose weight.
- Ⓑ to reduce the time that weight-loss interventions take.
- Ⓒ to promote greater understanding of the dangers of obesity.

39. Dr Jones now feels practitioners can justify weight loss interventions because

- Ⓐ patients are currently less sensitive about weight-related issues.
- Ⓑ a huge majority of patients feel that it is a suitable topic for discussion.
- Ⓒ circumstances demand that action is taken even if patients are reluctant.

40. When describing the trialling of the programme, Dr Jones says she was surprised by

Ⓐ the fact that most patients offered a place did actually join it.

Ⓑ the positive attitude of patients who accepted the treatment.

Ⓒ the number of patients who dropped out early.

41. From the outcomes of the trial we learn that

Ⓐ the results for all participants improved at a similar rate.

Ⓑ intervention was consistently more successful than non-intervention.

Ⓒ the advantages of intervention became less marked in the longer term.

42. What does Dr Jones feel are the implications of the findings of the trial?

Ⓐ Areas other than weight loss may be suitable for similar initiatives.

Ⓑ Doctors may find that their role in the community starts to change.

Ⓒ There could be widespread benefits if they were applied nationally.

That is the end of Part C.

You now have two minutes to check your answers.

THAT IS THE END OF THE LISTENING TEST

BLANK

Patient	**Mrs Wayne**
Condition history	• diagnosed arthritis in hands and wrists • very swollen **(1)**Answer • pain makes simple everyday tasks difficult • problems when she has to **(2)**Answer things and move her wrist
Lifestyle details	• patient is 40 years old • previously worked as a **(3)**Answer • now does volunteer work at a primary school and on a helpline - uses a hands-free phone
Impact on daily life	• difficult getting in and out of bed or a bath or a low chair • uses a walking aid and a **(4)**Answer • has a **(5)**Answer
Previous treatment	• heat pads
Patient's treatment requests	• **(6)**Answer • check effectiveness of **(7)**Answer
Other previous treatment	• saw an **(8)**Answer ◦ helped patient with common tasks ◦ provided ideas to reduce **(9)**Answer
Medication	• mainly takes analgesics for pain • has stopped taking **(10)**Answer ◦ not effective ◦ made patient nauseous • occasional **(11)**Answer • also uses an **(12)**Answer (twice daily)

Patient	Roy Dafnis
Patient's explanation about condition	• is worried about his (1)Answer _____ while he's in hospital
History of condition	• can be brought on by ○ feeling (2)Answer _____ ○ some (3)Answer _____
Current condition	• (4)Answer _____ worse recently • possibly exacerbated by (5)Answer _____ of operation • is on his face, (6)Answer _____ • no sores or weeping • most concerned about (7)Answer _____
Management of condition	• currently having (8)Answer _____ • uses a (9)Answer _____ • wears appropriate clothing • hourly use of an (10)Answer _____ • only uses corticosteroids as a last resort • has seen a dermatologist for treatment • currently on an (11)Answer _____ • has helped with the scratching and (12)Answer _____

Part-B

1-You hear a physician talking to a group of trainee doctors.

What is the reason for the presentation about honeybees?

Select one:

- A. To advise what to do when a person is stung.
- B. To explain the reasons why people get stung.
- C. To illustrate how dangerous bees can be.

2-You hear two dentists discussing a patient.

What do they agree?

Select one:

- A. To limit further complications for the patient.
- B. The patient has a record of poor oral hygiene.
- C. To seek advice about the patient's medication.

3-You hear an optometrist giving a presentation to a group of teachers.

He says that students should be tested for colour blindness

Select one:

- A. to improve class performance.
- B. regardless of their gender.
- C. by a qualified health professional.

4-You hear a radiographer talking to a patient about their CTC examination.

The radiographer is explaining

Select one:

- A. how to prepare for the examination.
- B. who to contact in order to cancel the examination.
- C. why certain equipment is used during the examination

5-You hear a physiotherapist speaking to a support group for Parkinson's disease.
She says that physiotherapists help patients by
Select one:

- A. using a variety of approaches.
- B. assessing their overall health issues.
- C. working with other medical professionals.

6-You hear an oncologist talking with a patient about their test results.
The patient is anxious because she
Select one:

- A. doesn't understand her treatment options.
- B. still has stomach pain.
- C. may have cancer.

PART-C

1-You hear a presentation on asbestos by a respiratory physician called Dr Bill Musk.

Dr Musk suggests the use of asbestos was widespread because it was

Select one:

- A. a comparatively low-cost product.
- B. at one time considered safe to use.
- C. able to be sourced throughout the world.

2-Despite asbestos being banned in 2003, Dr Musk suggests people still present with asbestos-related diseases because

Select one:

- A. it takes a long time for symptoms to appear.
- B. asbestos is still found in many places.
- C. asbestos fibres are difficult to see.

3-Dr Musk mentions the research conducted on asbestos to highlight that

Select one:

- A. the substance itself can take many forms.
- B. everyone's reaction to contact with it is different.
- C. the difficult effects it has on individuals who have contact with it.

4-Dr Musk says that people most at risk from exposure to asbestos

Select one:

- A. sometimes have other health issues.
- B. are categorised by their profession.
- C. have the potential to affect others.

5-Dr Musk discusses asbestos-related diseases from a US study to

Select one:

- A. highlight how many people have died.
- B. explain their different outcomes for patients.
- C. emphasise the effects of exposure to the fibres.

6-Dr Musk explains that treatment of asbestos-related diseases

Select one:

- A. works best when symptoms are recognised early on.
- B. doesn't have its own particular management.
- C. has a relatively high success rate.

E2:

1-Dr Wallace explains that parasomnia sleep disorders

Select one:

- A. have been known to cause conflict between couples.
- B. are closely linked to the way a person dreams.
- C. means the patient has an underlying issue.

2-Dr Wallace suggests that when establishing a diagnosis of parasomnia treating physicians should

Select one:

- A. identify if there is a family history
- B. consider the patient's physical and mental health.
- C. ensure the patient understands what is happening.

3-Dr Wallace explains that for sufferers of parasomnia the outlook

Select one:

- A. gets worse as they get older.
- B. is dependent on their mental health.
- C. can vary according to their particular condition.

4-When recounting the case with Marc, Dr Wallace suggests that

Select one:

- A. there were no reasons for the signs he was displaying.
- B. his sleep disturbances were related to anxiety.
- C. the incidents were affecting his memory.

5-Dr Wallace says that Marc's 2 minor arousals led to a diagnosis of sleep terror because

Select one:

- A. it's rare for someone to have them during a sleep study.
- B. he appeared to wake up and look around the room.
- C. they had ruled out all other possible options.

6-Dr Wallace's follow-up advice and treatment of Marc suggests that he

Select one:

- A. had trouble coping when his parents weren't with him.
- B. was dependent on medication to help him sleep.
- C. would outgrow the insecurity he was feeling.

Patient	**David Johnston**
Patient's explanation about condition	• **(1)**>>[ ] - 1 week ago • no initial pain or other symptoms • began feeling **(2)**>>[ ] a few days later • diagnosed with **(3)**>>[ ]
Current symptoms	• last day or two - **(4)**>>[ ] • still feeling dizzy • also feels **(5)**>>[ ] • experiencing fatigue
Medical history	• **(6)**>>[ ] allergy • flu last winter • **(7)**>>[ ] - sports injury
Family medical history	• **(8)**>>[ ] • **(9)**>>[ ] - parents and grandparents • grandfather - stroke • possible **(10)**>>[ ] in grandfather
Patient's concerns	• wants to play in **(11)**>>[ ] next week
Advised patient	• not to play (possible serious consequences) • to rest and **(12)**>>[ ]

Patient	**June Wilcox**
Patient's description of condition	• problems breathing **(1)**>>_____ • has felt **(2)**>>_____ (gave up netball) • worse at night • started **(3)**>>_____ 12 - 18 months ago • serious fatigue in the morning
Previous treatment	• **(4)**>>_____ from the chemist (not effective long-term) • GP prescribed **(5)**>>_____
Diagnosis	• positive **(6)**>>_____ • accompanied by **(7)**>>_____
Suggested course of action	• recommended **(8)**>>_____ to align cartilage structure
Patient's concerns	• worried about **(9)**>>_____ ◦ previous complications with **(10)**>>_____ ◦ offered to outline **(11)**>> _____ options • sometimes suffers with **(12)**

Part-B

You hear a nurse and a senior nurse discussing a patient.

1-Where is the patient's range of motion most restricted?

Select one:

- A. in his left shoulder.
- B. in his right knee.
- C. in his hips.

2-You hear an optometrist talking about amblyopia to parents and teachers at a kindergarten.

What is he optometrist doing?

Select one:

- A. explaining the effects of the condition.
- B. recommending children get tested.
- C. outlining treatment options.

3-You hear a nursing lecturer talking about side effects to a group of student nurses.

What is she discussing?

Select one:

- A. the medications with the greatest amount of reactions.
- B. the types of patients who experience reactions to medication.
- C. the reactions some people experience to some common medications.

4-You hear a senior registrar talking about cholesterol to a group of junior doctors.

What is he doing?

Select one:

- A. highlighting the dangers of doing nothing.
- B. explaining when treatment may be required.
- C. suggesting that it can be difficult to make a diagnosis.

5-You hear an agency nurse checking his schedule with an employee at the agency.

When does the agency employee need the nurse to work?

Select one:

- A. on Saturday.
- B. on Monday.
- C. on Friday.

6-You hear a conversation between a senior nurse and a student nurse.

The senior nurse is

Select one:

- A. explaining a range of safety measures.
- B. highlighting how easily accidents can happen.
- C. outlining the use of some important hospital equipment.

Finish attempt ...

Part-C

1-Ms Parr has raised the issue of belly fat because

Select one:

- A. more education is required.
- B. it is affecting so many people.
- C. experts can't agree on its causes.

2-Why does Ms Parr believe belly fat is so dangerous?

Select one:

- A. Most people have more of it than they used to.
- B. There are currently no medications to treat it.
- C. It causes a complex response in the body.

3-What are Ms Parr's views about losing body fat?

Select one:

- A. Overall reduction is the most important factor.
- B. Targeting high risk areas is the best approach.
- C. There are good new methods to help lose weight.

4-Ms Parr considers a healthy diet to be one that

Select one:

- A. avoids all types of sugar.
- B. encourages burning food as fuel.
- C. contains all the bodies' essential nutrients.

5-Ms Parr says that some people have trouble losing fat because of

Select one:

- A. their failure to accept they have a problem.
- B. social factors that are difficult to control.
- C. the pressures associated with a busy lifestyle.

6-When does Ms Parr recommend people seek further advice?

Select one:

- A. If they are a lot heavier than they used to be.
- B. If they aren't as strong as they once were.
- C. If they notice they have a wide waistline.

E2

1-Why does Dr Richter mention the numbers related to deaths from suicide?

Select one:

- A. to suggest that better prevention techniques are needed.
- B. to indicate where most of the problem is located.
- C. to highlight how many people it affects.

2-Dr Richter believes the most important area for suicide prevention is

Select one:

- A. understanding where a patient falls within specific classifications.
- B. the doctor’s role in judging a patient’s particular condition.
- C. the patient taking responsibility for their current predicament.

3-What does Dr Richter say helps when identifying suicidal risk?

Select one:

- A. Considering specific views from the patient themselves.
- B. Acknowledging a patient’s potential mental health issues.
- C. Accepting that each patient has their own unique problems.

4-What are Dr Richter’s views on antidepressants being used in suicide prevention?

Select one:

- A. they should only be used with older patients.
- B. they are not as well understood as they should be.
- C. they need to be more effectively integrated with other actions.

5-Regarding the risk of suicide, Dr Richter suggests that

Select one:

- A. the need for increased levels of medication.
- B. governments could be doing a lot more to help.
- C. modern-day society is a major contributing factor.

6-Dr Richter explains that to improve secondary suicide prevention in the future

Select one:

- A. there needs to be attention given to innovative methods.
- B. greater emphasis needs to be placed on physician training.
- C. detailed analysis of what does and doesn’t work needs to occur.

Patient	**Joseph Alvarez**
Recent medical history	• blood in urine • sharp **(1)**... • clear ECG and ultrasound of **(2)**... • normal **(3)**... - 2 years ago
Current symptoms	• stomach constantly **(4)**... - past 18 months • good day - no pain/bad day - constant pain • sleeps badly due to **(5)**... • experiences pain before eating - pain returns 30 mins later • **(6)**... food makes it worse
Medications	• has never **(7)**... • bitter, sour taste when **(8)**... • used to take **(9)**... for an ulcer
Lifestyle details	• drinks a few glasses of wine per week • used to smoke **(10)**... a day - quit recently
Family medical history	• mother - died from a heart attack aged 90 • father - died from **(11)**... at age 77
Patient's concern	• worried it could be **(12)**...
Patient	**Julie Dooley**

Reason for presenting

- breast lump
  - first noticed **(1)**...
  - located in **(2)**...

Family history

- six months ago **(3)**... from breast cancer (age 65)
- no other relevant history?

Personal details

- patient **(4)**... 9 months ago
- 2 children
  - daughter aged 14
  - son aged 12

Medical history

- nil reports
- nil **(5)**...

Other symptoms

- recent pain in chest - feels like a **(6)**...
- pain radiates to **(7)**...
- caused patient to **(8)**... on one occasion
- no other symptoms

Physical examination

- located a **(9)**... lump
- approximately **(10)**... long

Patient's concerns

- fearful of having a **(11)**...
- prepared to undergo **(12)**...

PART B

1-You hear a junior doctor speaking with a patient in the cardiac clinic while a consultant is present.

What does the patient suggest about their pain?

Select one:

- A. it affects his breathing.
- B. it is located mainly in his chest.
- C. it makes it difficult for him to go walking.

2-You hear a senior nurse talking about removing sutures to a group of students.

What is she doing?

Select one:

- A. Recommending a clear procedure to follow.
- B. Explaining why a clean environment is important.
- C. Suggesting that sometimes there may be complications.

3-You hear a staff nurse talking to a patient she is escorting to the Radiology Department.

What does she need to know from the patient?

Select one:

- A. If he is ready.
- B. How he is feeling.
- C. His personal details.

4-You hear a nurse talking with a patient in the recovery unit.

What is the patient worried about?

Select one:

- A. Her breathing difficulties.
- B. The pain in her chest.
- C. How cold she feels.

5-You hear the beginning of a training session for doctors.

The trainer explains that the session will focus on

Select one:

- A. how hormones work.
- B. the purpose of various glands.
- C. problems with the endocrine system.

6-You hear a doctor talking to a nurse about a patient called Mr Kransky.

The doctor wants the nurse to

Select one:

- A. ensure the patient can feed themselves by the end of the week.
- B. contact the OT about the patient's swallowing reflex.
- C. continue the patient on their current diet.

Part-C

1-Dr Werner suggests that when it comes to vitamin D production

Select one:

- A. the sun is important.
- B. too much sun can be dangerous.
- C. without the sun we increase our chance of illness.

2-What does Dr Werner say about the benefits of vitamin D?

Select one:

- A. It eliminates a lot of serious disease.
- B. More health practitioners should be supporting it.
- C. There are a lot of unsubstantiated theories about it.

3-Dr Werner mentions the benefits of sun protection in order to

Select one:

- A. highlight the importance of covering up.
- B. provide statistics on the incidence of cancer.
- C. discuss the different types of energy produced.

4-Dr Werner believes using sun protection doesn't lead to vitamin D deficiency because

Select one:

- A. most people use low-SPF sunscreens.
- B. it is impossible to block all ultraviolet light.
- C. radiation levels are higher than they used to be.

5-According to Dr Werner, we don't need very much sun exposure because

Select one:

- A. vitamin D deficiency is very rare.
- B. too much vitamin D can cause problems.
- C. even small quantities provide enough vitamin D.

6-Dr Werner believes everyone can acquire the necessary allowance of vitamin D from

Select one:

- A. a combination of food and supplements.
- B. dietary supplements alone.
- C. certain types of food.

E2

1-Roger believes radiation therapists are necessary because

Select one:

- A. there are so many different types of cancer.
- B. patients need help when battling cancer.
- C. it is an essential way to treat cancer.

2-What led Roger into becoming a radiation therapist?

Select one:

- A. He was inspired by the man who treated his mum.
- B. He wanted to save people after his mum died from cancer.
- C. He learnt so much while his mum was dying and wanted to put it to use.

3-Roger says that when treating patients, radiation therapists

Select one:

- A. often work very long hours.
- B. regularly use a range of approaches.
- C. need to be aware of dangerous emission levels.

4-Roger mentions the case of his former patient, Susan, to

Select one:

- A. show that not everyone feels the same about radiation therapy.
- B. highlight the types of equipment used in radiation therapy.
- C. explain the steps involved in radiation therapy.

5-Why does Roger find teaching radiation therapy rewarding?

Select one:

- A. He likes knowing his students are making a difference.
- B. There is a lot that that he has learnt that he can pass on.
- C. It provides opportunities for him to work with past patients.

6-What does Roger suggest about cancer treatment in the future?

Select one:

- a. Further research is needed to reduce instances of the disease.
- b. More patients should be encouraged to talk about their condition.
- c. Physicians need to do a better job of reducing patient stress.

Patient	**Mrs Black**
Patient's reasons for presenting	• family recently moved to the area • concerned daughter **(1)**-_____
Details about daughter	• Name: Mia • Age: **(2)**-_____
Mia's eating habits	• doesn't eat **(3)**-_____ anymore • eats in **(4)**-_____ • skips **(5)**-_____ often • change in eating habits began last year
Mia's school situation	• good student but **(6)**-_____ • has been having trouble **(7)**-_____ lately • has a few close friends but not boyfriend • mum thinks Mia is concerned about **(8)**-_____
Mia's physical health	• does ballet • lies around in her room • mum reports Mia has **(9)**-_____ recently • weighs herself a lot • looks at herself in the mirror • doesn't sleep well (tired and moody) • sometimes has headaches and has taken **(10)**-_____ • may have been **(11)**-_____
Doctor's advice for Mrs Black	• schedule follow-up appointment with Mia and parents • possible referral to a **(12)**-_____

Patient Ryan Henderson

Patient's description of symptoms

- pain located in the left knee **(13)**-
- pain began 4 weeks ago (noticed first when running)
- described pain as **(14)**-
- worse going from sitting to standing position or when sitting with a **(15)**-
- worse **(16)**- in the day
- doesn't wake patient at night
- no other symptoms noted (pins, and needles, numbness)

Medical history

- **(17)**-left knee 15 years ago
- scare still visible (no associated discomfort)
- broke little toe on left foot aged 10
- broke fibula in **(18)**- aged 25
- broke little finger on right hand last year
- last year bursitis in **(19)**-

Previous treatment

- **(20)**- for bursitis
- stopped running for 6 months
- did stretching and **(21)**- two times a week
- treatment helped - started running again last Xmas

Physical examination findings

- niggly pain in front knee cap standing on left leg
- half squat caused pain in **(22)**-
- pain in left knee worse after half squat (sharp pain)
- left knee felt **(23)**- on strength testing

Diagnosis

- left knee-cap appears to be misaligned
- iliotibial band is tight

- **(24)**-  on outside of left knee (underlying inflammation)

Part-B

1-You hear a doctor talking to a patient who has been having trouble with his water.

The doctor wants to confirm if the patient

Select one:

- A. has had burning when passing water.
- B. has a blockage in their kidneys.
- C. has high blood pressure.

2-You hear a nurse briefing her colleague about a patient.

What does she want her colleague to do?

Select one:

- A. Check the patient is receiving the correct amount of fluids.
- B. Order another chest radiograph to check for an effusion.
- C. Confirm the patient is stable and check his blood culture.

3-You hear part of a consultation at a GP practice with a patient suffering from depression and sleep disturbances.

The GP recommends cognitive behavioural therapy because

Select one:

- A. there are better ways for the patient to respond to life's challenges.
- B. it will help the patient better manage the way he feels when stressed.
- C. the patient needs to control his anger and medication isn't working.

4-You hear a nurse seeking advice from a doctor.

Why does the nurse telephone the doctor?

Select one:

- A. She would like to know how to proceed with the patient.
- B. The patient is in intense pain and has a fever.
- C. She suspects that the patient has an infection.

5-You hear the principal of an aged care home talking to staff about osteoporosis.

She wants to highlight that

Select one:

- A. there are now treatments available for men.
- B. residents are not getting enough calcium and vitamin D.
- C. a lot of the residents may already have osteoporosis.

6-You hear two nurses discussing a patient, Mrs Robbins.

Julie says Mrs Robbins refused to mobilise because

Select one:

- A. she was happy having a sponge bath.
- B. she wanted to hear directly from the doctor.
- C. she was upset and wanted to be left alone.

Part-C

1-Dr Tredellow believes snoring poses concerns because

Select one:

- A. it is possible for anyone to become a snorer.
- B. it contributes to several other health issues.
- C. everyone has an upper airway that collapses.

2-Dr Tredellow says the main cause of snoring is

Select one:

- A. drinking too much.
- B. being overweight.
- C. a narrow airway.

3-According to Dr Tredellow there may be societal impacts from snoring for a person who

Select one:

- A. is suffering from obesity.
- B. has concentration issues.
- C. is worried about their snoring.

4-What point does Dr Tredellow make about obstructive sleep apnoea?

Select one:

- A. snoring sometimes causes people to stop breathing.
- B. snoring can be linked to obstructive sleep apnoea.
- C. obstructive sleep apnoea and snoring are similar.

5-Dr Tredellow feels research on carotoid artery atherosclerosis

Select one:

- A. is still in its infancy despite highlighting some serious issues.
- B. shows that snoring may be far more dangerous than once believed.
- C. demonstrates a clear connection between snoring and heart disease.

6-What does Dr Tredellow say about the treatment for snoring?

Select one:

- A. Making specific lifestyle changes contributes to a reduction in snoring.
- B. There are too many causes of snoring to be able to treat them all.
- C. Treatment is best provided by a recognised sleep physician

E2

.

1-Why does Dr Munro say he volunteered to treat Ebola patients in Liberia?

Select one:

- A. because of the high death rate from Ebola in Africa.
- B. because medical treatment in Africa can be aggressive.
- C. because he believed he had a duty to try and help.

2-Dr Munro says the patient's he treated were often afraid because

Select one:

- A. there was a high death rate for patients who received treatment.
- B. family members didn't believe that treatment was helping.
- C. they didn't trust the doctors because they were foreigners.

3-What does Dr Munro say about the specialised clothing he had to wear?

Select one:

- A. That he spent more time rehydrating than treating patients.
- B. That patients found it similar to what is worn in a hospital.
- C. That it affected the way he would treat his patients.

4-Why does Dr Munro believe the case of the young girl is tragic?

Select one:

- A. It highlights how even young children can get Ebola.
- B. It demonstrates how families were affected by the disease.
- C. It is a situation that only affected a small group of patients.

5-Treating Ebola patients in Liberia has made Dr Munro

Select one:

- A. improve how he manages the emotional needs of his patients.
- B. realise that seeing patients suffering is part of a doctor's job.
- C. understand that doctors can't do everything for their patients.

6-Dr Munro believes Ebola may spread to other parts of the world because

Select one:

- A. it takes longer for the disease to develop than people think.
- B. plane travel has made the world a much smaller place.
- C. an infected person in a crows could spread the disease.

Patient	**Mr Lajowski**
Details about procedure	• removal of lump from back • lump located near **(1)**- • worried about **(2)**- after operation.
Medical history	• has **(3)**- • duration - approximately 5 years
Physical problems	• trouble **(4)**- • falls over sometimes • sometimes freezes in one spot (akinesia) • **(5)**- shake a lot • **(6)**- arms
Problems with daily activities	• **(7)**- in bed • reaching for things • turning around when **(8)**- from a chair • tying shoelaces • doing and undoing **(9)**- • untidy **(10)**-
Other issues	• cramps at night - 12 months • patient's **(11)**- is affected • recently changed medication - cramps **(12)**-

Patient	**Chloe**
6 months ago	Valium from GP
4 months ago	**(1)**-[ ] began
3 weeks ago	discharged from hospital
Patient's description of symptoms	• felt like she was going to fall over • **(2)**-[ ] • trouble walking • heavy legs • palpitations • possible **(3)**-[ ] • became **(4)**-[ ] • heavy arms • difficult to **(5)**-[ ] things • **(6)**-[ ] in arms
Possible causes	• busy lifestyle • possible underlying **(7)**-[ ]
Other symptoms	• early stage **(8)**-[ ] in hands
Personal history	• married for two years • husband - steel fitter (own business) • fights about keep the **(9)**-[ ] • have seen a gynaecologist **(10)**-[ ] twice • waiting on husband's test results • returning in **(11)**-[ ]
Recommendations	• prescribed **(12)**-[ ] for anxiety • follow-up appointment in two weeks

PART-B

1-You hear a doctor talking to the daughter of a patient from an aged care home.

The doctor wants to

Select one:

- A. ensure that Mrs Cox's father has a good quality of life.
- B. advise Mrs Cox that her father has Alzheimer's disease.
- C. explain the improvements in medical treatment to Mrs Cox.

2-You hear a consultant giving a talk to a group of trainees.

What is the purpose of the talk?

Select one:

- A. to highlight the symptoms associated with a head injury.
- B. to discuss the importance of waking patients after a head injury.
- C. to convey the significance of the initial 24 hours after a head injury.

3-You head a vet talking to a client who brought this pet cat, Jimmy, to the vet clinic.

What does the vet say has caused Jimmy's problem urinating?

Select one:

- A. Stress from renovations.
- B. A fatal urethral obstruction.
- C. Struvite cyrstals in his urethra

4-You hear the nurse unit manager (NUM) discussing the progress if a patient at a multi-disciplinary team meeting.

What does she indicate about the patient's son?

Select one:

- A. That he hasn't been feeding the patient.
- B. That the patient is largely dependent on him.
- C. That he is having problems in his role as a carer.

5-You hear a radiographer talking to nursing staff at an aged care centre.

The radiographer is

Select one:

- A. helping nurses prepare patients for CT scans.
- B. stressing the importance of contrast media in CT scans.
- C. highlighting problems that occur when conducting CT scans.

6-You hear a GP discussing psoriasis with a patient.

The GP recommends sunbed treatment because

Select one:

- A. UVB light is unique in a hospital setting.
- B. there is currently a shortage of cream.
- C. it will help with the effects of the rash.

Part-C

1-What is the main focus of Sally's talk on tetanus?

Select one:

- A. It is impossible to avoid tetanus bacteria.
- B. It is essential to be vaccinated against tetanus.
- C. It is important to develop natural tetanus immunity.

2-Sally believes that deaths from tetanus still occur because

Select one:

- A. the bacteria is about to infect a person through the tiniest of wounds.
- B. infected people don't seek medical treatment early enough.
- C. not enough people are immunised against tetanus.

3-Why do some people believe the tetanus vaccine isn't necessary?

Select one:

- A. Tetanus bacteria can't enter a person's bloodstream.
- B. Tetanus bacteria are everywhere, giving people natural immunity.
- C. Tetanus bacteria aren't affected by medication or vaccination.

4-Sally suggests the woman didn't understand the dangers of tetanus because

Select one:

- A. she hasn't undergone a recent vaccination.
- B. she sought medical treatment very late.
- C. she ignored many early warning signs.

5-Sally highlights the case of the 4-year-old boy to

Select one:

- A. explain the symptoms of tetanus infection.
- B. show how tetanus can affect small children.
- C. illustrate how difficult it is to diagnose tetanus.

6-Why does Sally believe the man's case highlights how everyone is at risk of tetanus?

Select one:

- A. Because he arrived at hospital with no health complaints.
- B. Because the man had no injuries, but he still had tetanus.
- C. Because he couldn't remember when he was immunised

WINTER OLYMPIC GAMES

1-Dr Mene suggests that her role as CMO required her to

Select one:

- A. use a range of care options to assist athletic performance.
- B. work longer hours than usual to manage all the injuries.
- C. exercise sympathy because it was busy for everyone.

2-Dr Mene says the medical support of athletes

Select one:

- A. was tailored to the specific needs of the individual.
- B. was the same as treating patients at a medical clinic.
- C. depended on the type of health care provider available.

3-What is Dr Mene's view on providing care for life-threatening injuries?

Select one:

- A. A level of team work would have ensured a positive outcome.
- B. They knew there wouldn't be any to attend at the Olympics.
- C. If they had occurred, flexible treatment would have been required.

4-Dr Mene says her team prevented injuries by

Select one:

- A. reducing the amount of competition and travel for each athlete.
- B. evaluating what athletes ate and what they did after competing.
- C. having more advanced technology than other competing nations.

5-Dr Mene was impressed with he software they used because

Select one:

- A. it was particularly useful for musculoskeletal injuries.
- B. it had tremendous communication potential.
- C. it can help in critical care situations.

6-Dr Mene thinks any physician wanting to treat athletes at a major sporting event should

Select one:

- A. understand that you won't make a lot of money doing it.
- B. apply what they learn at these events to their private practice.
- C. take the time to forge connections that enhance your reputation.

Patient	**Miss Wells**
Background details	• 22 years old • lives with partner • never been pregnant • normal (1)*[ ] 3 months ago

Condition history

Feburary	• sharp stomach pain **(2)***[ ] • started a few days before period • ended when period finished
After Feburary	• constant pain - **(3)***[ ] • **(4)***[ ] performed • endometrosis **(5)***[ ] and on left ovary • took the pill without break • prescribed progesterone – weight gain, bloating and **(6)*** [ ]
May	• readmitted to hospital • **(7)***[ ] treatment • pain significantly reduced
August	• pain returned – condition similar to February • pain at different times of month • significant pain during **(8)***[ ]
Medical history	• regular periods - most recent **(9)***[ ] • never smoked • drinks at weekends - 1 or 2 drinks

- no **(10)*** _____ in the family
- regular bowel movements

Patient's concerns

- wants **(11)*** _____
- **(12)*** _____ affecting her mood

Patient

Mr Georges

Consultations with Mrs Georges, patient's wife

Background details

- Mrs Georges is feeling (1)* _____
- Mr Georges used to be very (2)* _____
- son lives close by
- also has a daughter

Patient history

- confused by common tasks
- has trouble (4)* _____
- he was initially (5)* _____ by his condition
- limited awareness now
- frequently repeated (6)* _____
- (7)* _____ very little now
- needs help when (8)* _____
- problem getting him to (9)* _____
- no longer interested in his appearance
- has recently started to (10)* _____ from house
- has trouble telling (11)* _____ apart
- sometimes plays with (12)* _____

Part B

1-You hear two doctors discussing a patient.

Dr Jones speaks to Dr Khan because he wants to

Select one:

- A. confirm his own diagnosis.
- B. hear Dr Khan's diagnosis.
- C. send the patient for tests.

2-You hear a dietician talking to a patient.

What is she doing?

Select one:

- A. Showing the patient alternative diet plans that eliminate fat.
- B. Explaining why the doctor wants the patient to eat a low-fat diet.
- C. Empathising with the patient about the challenges of reducing fat.

3-You hear a professor of emergency medicine giving a presentation to a group of trainee doctors.

The presentation is about

Select one:

- A. why European wasps are a problem.
- B. what European wasps like to eat.
- C. how European wasps sting.

4-You hear a GP talking to a regular patient who has been having kidney problems.

What does the GP suggest?

Select one:

- A. That the patient’s lifestyle will change forever.
- B. That there is more than one way to treat the patient.
- C. That the patient needs dialysis for the rest of their life.

5-You hear a physiotherapist in a hospital talking with John, a new patient.

The physiotherapist is

Select one:

- A. providing sports recovery options.
- B. explaining why warm showers are beneficial.
- C. giving instructions to assist with recovery from an injury.

6-You hear a specialist physician and a nurse discussing a patient’s treatment.

The nurse is unable to help the physician because

Select one:

- A. she isn't senior enough
- B. she isn't familiar with the patient.
- C. she doesn't understand the treatment options.

Part C

1-What does Dr Dugan suggest defines a patient's health literacy?

Select one:

- A. how frequently they use available health services.
- B. how well they communicate with health professionals.
- C. how clearly they understand health-related information.

2-As a result of her own research, and other studies, Dr Dugan believes

Select one:

- A. most patients don't listen carefully enough to health professionals.
- B. patient health outcomes rely heavily on a person's health literacy.
- C. governments aren't doing enough to assist with health literacy.

3-Dr Dugan says most health professionals have at one time

Select one:

- A. had complaints made about their poor communication.
- B. incorrectly assumed their instructions were understood.
- C. forgotten how to speak to patients in their own language.

4-Why does Dr Dugan share the examples about previous patients?

Select one:

- A. to show how she teaches health literacy.
- B. to illustrate findings from recent research studies.
- C. to demonstrate the importance of clear communication.

5-The paediatrician example about the "teach-back" method demonstrates

Select one:

- A. how easy it is to overlook whether or not a patient has understood instructions.
- B. what patients do and do not understand about health literacy.
- C. the best way to confirm what a patient understands.

6-What does Dr Dugan make about improving patient safety and care?

Select one:

- A. It is essential to have reliable health care for all patients.
- B. Focusing on acute care settings is the most important aspect.
- C. Delivery of information beyond the traditional care settings is vital.

E2

1-Dr Marshall believes that the low-fat era is coming to an end because

Select one:

- A. the US Dietary Guidelines have changed towards fat.
- B. 35 years ago the US Dietary Guidelines were very different.
- C. fat and cholesterol are no longer linked in the US Dietary Guidelines.

2-Dr Marshall suggests that LDL particles

Select one:

- A. are more dangerous when they’re smaller.
- B. have nothing to do with saturated fat.
- C. pose no risk to a person’s health.

3-What are Dr Marshall's views on saturated fats?

Select one:

- A. Refined carbohydrates are more dangerous than saturated fat.
- B. Not all saturated fats are the same as others.
- C. Vegetable oil should replace saturated fat.

4-Why doesn't Dr Marshall believe people should stop worrying about cholesterol?

Select one:

- A. Biological solutions haven’t been able to solve the problem.
- B. People continue to eat too many foods high in fat and sugar.
- C. More research is required to understand how fat enters the blood.

5-Dr Marshall says there is no agreement over nutritional science because

Select one:

- A. there are too many opinions when it comes to a person’s diet.
- B. there are a lot of different ways to interpret the information.
- C. there isn’t enough money to fund high quality research.

6-In Dr Marshall's opinion, many organisations and experts need to

Select one:

- A. become more open-minded about embracing current studies.
- B. encourage more people to eat a diet high in vegetables.
- C. stop saying that eating saturated fat is bad.

OCCUPATIONAL ENGLISH TEST

LISTENING SUB-TEST – QUESTION PAPER

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES: Your details and photo will be printed here.

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: APPROXIMATELY **40 MINUTES**

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper.**

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part A: Write your answers on this **Question Paper** by filling in the blanks.

Example: Patient: Ray Sands

Part B & Part C: Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

Ⓐ
Ⓑ
Ⓒ

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Extract 1: Questions 1-12

You hear a cardiologist talking to Sarah, a patient with a pacemaker. For questions 1-12, complete the notes with a word or short phrases.

You now have 30 seconds to look at the notes.

Patient: Sarah

Sarah's medical history:

1. Had surgery in _____ to correct her heart problem.

2. Her parents tried to explain that she had _____

History of surgery and Sarah's memories:

3. It was _____ after surgery.

4. Her lips and nails were _____ .

Patient's exercise and fitness:

5. She kept _____ consistently as she was getting older.

6. She used to dance for _____ a Week.

7. She is still _____ regularly.

8. She gets _____ every now and then.

Drinking habits and heart condition:

9. She has _____ drinks once a week.

10. Has _____ chest pain.

11. Noticed _____ easily.

12. Her family history of heart diseases is _____

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Extract 2: Questions 13-24

You hear a GP talking to Kevin about his accident. For questions 13-24, complete the notes with a word or short phrases.

You now have 30 seconds to look at the notes.

Patient: Kevin

Patient's general medical history, exercise and lifestyle:

13. He is _____ years old.

14. His _____ is lower than average.

15. He _____ for about 100-150 kilometers a week.

Family medical history:

16. His mother is a _____ .

17. His father had a _____ .

Kevin's health problems and medical treatment in West Africa:

18. Had a lot of _____ troubles.

19. Had _____ constantly.

20. He received _____ medical care .

Patient's bicycle accident and onset of pain:

21. Smashed himself on _____ .

22. After the accident they went to the _____

23. Pain started after having _____

24. Pain appeared after _____ level went down.

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Part B

In this part of the test, you will hear six different extracts. In each extract, you will hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A, B** or **C**) which fits best according to what you hear. You will have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

25. Which of the following information is INCORRECT regarding the patient's condition?

A – His dose of morphine has been increased

B – He is on Amoxi-clav

C – He takes hypertension medication

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26. According to the brief given by the nurse

A – It took 10 mins of CPR to reach Return Of Pulse

B – On arrival, the patient's O2 sat was 69%

C – He has a history of parathyroid cancer

27. According to te extract

A – The patient is 65 years old

B – Her presenting features were suggestive of pneumonia

C – The patient is to be commenced on antibiotics and steroids

28. According to the extract

A – The patient was diagnosed with lung cancer for the first time in 2009

B – When he arrived to the ER, his PO2 sat was 68%

C – The patient has a history of shortness of breath since one month

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29. Regarding the patient's analgesia

A – The pain is well controlled

B – The patient is not allowed to take analgesia anymore

C – Her last dose of analgesia was 3.30 mg

30. According to the extract

A – The patient is diagnosed as a case of stroke

B – His daughter reported that he was speaking irregularley

C – The patient is showing non-compliance regarding taking his antihypertensives

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear an interview with Professor Grobman, who's a *Professor of Obstetrics and Gynecology*.

You now have 90 seconds to read **questions 31-36**.

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31. What does professor Grobman believe to be the problem with induced labor?

Ⓐ That it inevitably leads to cesarean section.

Ⓑ That a belief was built on the basis of misunderstanding of certain results.

Ⓒ That the procedure has not been performed correctly.

32. The population undergoing induced labor are described as biased because

Ⓐ There was a greater number of women who underwent induced labor when compared with those who did not.

Ⓑ The difference in the gestational age between the two groups.

Ⓒ They had a medical problem that made inducing labor an indication.

33. What point does professor Grobman make by comparing induced to spontaneous labor?

Ⓐ Induced labor is optional.

Ⓑ Induced labor precedes cesarean section.

Ⓒ Spontaneous labor is safer.

34. What is the concern with regard to caesarean delivery truly about?

Ⓐ The patient being experiencing child birth for the first time.

Ⓑ The patient having co-morbid conditions.

Ⓒ The bad outcome when it comes to the babies health.

35. What would professor Grobman like to decrease the rate of?

Ⓐ Cesarean sections.

Ⓑ Vaginal deliveries.

Ⓒ Unnecessary interventions.

36. Women who were randomized to normal vaginal delivery had …….. outcomes when compared to those going through induced labor

Ⓐ worse.

Ⓑ better.

Ⓒ the same.

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Extract 2: Questions 37-42

You hear an interview with **Serge Negus** and **Kathleen Bainbridge** , about Phantom smells.

You now have 90 seconds to read **questions 37-42**.

37. Serge states that he lost his sense of smell as a result of

Ⓐ a fractured skull.

Ⓑ a broken nose.

Ⓒ nerve damage.

38. Why didn't Serge mind experiencing phantom smells?

Ⓐ The experience wasn't that bad.

Ⓑ He realized that it is common and could happen to anyone.

Ⓒ It is untreatable anyway.

39. What is Serge telling us about the slightly bad smell that he had?

Ⓐ he couldn't find the place where it is coming from.

Ⓑ It was unfamiliar to him.

Ⓒ He would choose never being able to smell again.

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40. According to Kathleen's definition of phantosmia, which of the following is true?

Ⓐ It is always perceived as of something is burning.

Ⓑ The patient is considered to be hallucinating.

Ⓒ It is not a true phenomenon.

41. What does Kathleen say about the study that she participated in?

Ⓐ They used batteries as the source of odour.

Ⓑ They included a question about phantom smells within the already established survey.

Ⓒ Over 7000 people reported experiencing phantom smells at least once.

42. What is the interviewer's opinion regarding the study?

Ⓐ Its results might be somehow inaccurate.

Ⓑ It was tested on a small number of subjects.

Ⓒ Its outcomes were surprisingly predictable.

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OCCUPATIONAL ENGLISH TEST

LISTENING SUB-TEST – QUESTION PAPER

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES: Your details and photo will be printed here.

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: APPROXIMATELY **40 MINUTES**

INSTRUCTIONS TO CANDIDATES:

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HOW TO ANSWER THE QUESTIONS:

Part A: Write your answers on this **Question Paper** by filling in the blanks.

Example: Patient: Ray Sands

Part B & Part C: Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

(A)
(B)
(C)

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Extract 1: Questions 1-12

You hear Rose, a neonatal intensive care nurse, talking to Peter about his newborn twin. For questions 1-12, complete the notes with a word or short phrases.

You now have 30 seconds to look at the notes.

Father's name: Peter

Initial details:

1. Twin babies born _____ Weeks early.

2. Olive is named after _____

Olive's current situation:

3. She is small and does not have a lot of _____.

4. She was taken to a _____ .

5. She does not have enough _____ fat to keep her warm.

Nurse's comment regarding the incubator stay:

6. They will start by giving fluids and a trial for _____ feed will be initiated .

7. She will get out when she can maintain her _____ .

8. No set weight or _____ .

History of pregnancy and Olive's condition:

9. They started scan by the _____week.

10. They missed a couple of scans because it was _____.

11. Olive's placenta contained less _____ .

12. Her oxygen saturation is _____ which is normal.

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Extract 2: Questions 13-24

You hear an optometrist talking to Michael, a man with macular degeneration. For questions 13-24, complete the notes with a word or short phrases.

You now have 30 seconds to look at the notes.

Patient: Michael

Patient's comments on his eyes and previous optometrist visits:

13. His last visit to the optometrist was _____ ago.

14. He purchased his most recent glasses _____ ago.

15. He has difficulty reading printed _____ books.

16. In 2006, he noticed _____ in reading lines.

17. Ophthalmologist referred him to a _____ specialist.

18. Specialist noticed _____ in front of retina.

19. Prescribed lucenitus _____ to be taken monthly.

20. He visits specialist every _____.

Patient's treatment and outcomes:

21. Doctors were _____.

22. He had _____ injections.

Optometrist's explanation of Amsler chart:

23. Most of the time, people do not use it _____

24. He needs to _____ one eye when using it.

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Part B

In this part of the test, you will hear six different extracts. In each extract, you will hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A, B** or **C**) which fits best according to what you hear. You will have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

25. Which of the following information is INCORRECT regarding the patient's condition?

A – He is on amiodarone

B – Has a wounded right sacrum

C – His glucose was high, so he received insulin

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26. What is this nurse's opinion regarding the new nurses

A – They want to be given everything without effort

B – They cannot manage 40 beds effectively

C – They want to work hard but they do not have the experience

27. According to te extract

A – The charge nurse had personal problems

B – The new nurse felt respected

C – They were both stressed at work

28. Regarding the patient's vital data

A – Oxygen saturation is 90%

B – Heart rate is 131 bpm

C – Respiratory rate is 11

29. According to the extract

A – Patient's condition is un-usual

B – The bleeding is resulting from the uterine contraction

C – The targeted oxygen saturation is 96%

30. According to the extract

A – The patient is shocked

B – The patient had been vomitting blood

C – The patient was started on blood transfusion

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Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear an interview with Professor John McNeil, who's an expert of preventive medicine.

You now have 90 seconds to read **questions 31-36**.

31. What important information does professor McNeil point up to regarding the study?

Ⓐ The increasing number of people taking Aspirin every morning.

Ⓑ The longer life-span of those administering Aspirin every morning.

Ⓒ That participants had no history of cerebrovascular events.

32. What does professor McNeil say regarding the results of the trials concerning the link between Aspirin and cancer?

Ⓐ More clinical trials are needed in order to come to an answer.

Ⓑ The exact effect is not yet confirmed.

Ⓒ It proves that people who take Aspirin are more liable to die of cancer.

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33. How does professor McNeil describe the course of bleeding as a complication?

Ⓐ Progressive.

Ⓑ Continuous.

Ⓒ Regressive.

34. What aspect of the study is considered important and needs further research?

Ⓐ Effects of Aspirin on the course of some diseases on the long run.

Ⓑ The fact that people who are overweight might require larger doses.

Ⓒ The similarity of odds regarding the development of dementia when comparing the two arms of the study.

35. The study was conducted over a period of five years to notice if the patient's age is related to

Ⓐ The long term complications.

Ⓑ Patients' compliance.

Ⓒ The influence of Aspirin.

36. Which of the following reasons does not require daily intake of Aspirin as to professor McNeil's opinion?

Ⓐ Upon doctor's advice.

Ⓑ prophylaxis from the occurrence of some diseases.

Ⓒ prophylaxis from the reoccurrence of some diseases.

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Extract 2: Questions 37-42

You hear an interview with Dr. **Miranda Davies-Tuck**, who's a perinatal epidemiologist.

You now have 90 seconds to read **questions 37-42**.

37. "**planned home-birth**" is a choice made by some women upon their

Ⓐ financial status.

Ⓑ personal preference.

Ⓒ previous experience.

38. Which of the following does not categorize a women as being at high risk?

Ⓐ high birth weighted babies.

Ⓑ fetal mal-presentations.

Ⓒ previous elective caesarian section.

39. In low risk women, The lower rate of morbidity in home delivered babies is a result of it being

Ⓐ well planned.

Ⓑ performed in a more friendly environment.

Ⓒ less likely to involve advanced medical equipment.

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40. What does Dr. Davie-Tuck mention as the reason behind the higher mortality rates in home deliveries of high risk women?

Ⓐ The co-morbid conditions.

Ⓑ The barriers that hinder their transfer to the hospital.

Ⓒ Not yet identified.

41. What is Dr. Davie-Tuck excited about regarding the studies in her country?

Ⓐ That they are mimicking those from international studies.

Ⓑ That they are growing in number and effectiveness.

Ⓒ That it helped a lot in the process of risk assessment.

42. Which of the following is a method that was used to overcome the barriers involving transferring high risk women to the hospital?

Ⓐ Providing more experienced midwives.

Ⓑ Establishing for a more appropriate midwife-obstetrician interaction.

Ⓒ Maintaining continuous after-birth follow-up.

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E2 LANGUAGE

Mock test

LISTENING PART A – QUESTION PAPER

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:
CANDIDATE SIGNATURE _____

Passport Photo

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper**.

DO NOT remove the OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part A: Write your answers on this Question Paper by filling in the blanks.

Example: Patient: Ray Sands

BLANK

Occupational English Test

Listening Test

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: --beep—

You'll have time to read the questions before you hear each extract and you'll hear each extract **ONCE ONLY**. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A

In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient.

For **questions 1-24**, complete the notes with information you hear.

Now, look at the notes for extract one.

Extract 1: Questions 1-12

You hear a GP talking to a patient called Daniel Anderson.
For questions **1-12**, complete the notes with a word or short phrase.
You now have 30 seconds to look at the notes.

Patient Daniel Anderson

Presenting Symptoms

- 2 wks / **(1)** _____ than usual
- Short of breath
- Cold/sore throat

Recent History

- Last **(2)** _____ months
- Ongoing condition
- Patient managed condition
- Bad sore throat and **(3)** _____
- Fatigue
- Shortness of breath – becoming more **(4)** _____
- Breathing normal **(5)** _____
- No history **(6)** _____ /chest problem
- Non-vegetarian
- Describes diet as **(7)** _____
- Low fruit and vegetable intake
- Drinks **(8)** _____ of coffee a day
- Ongoing mild indigestion
- Bowel Function – **(9)** _____

Family History

- No family history **(10)** _____
- Parents - healthy
- **(11)** _____ – no known health problems
- Patient suspects **(12)** _____

Extract 2: Questions 13-24

You hear a GP talking to a patient called Mrs Wright. For **questions 13-24**, complete the notes with a word or short phrase.
You now have thirty seconds to look at the notes.

Patient Mrs Wright

Background

- Follow up consultation
- Patient has **(13)** _____
- Initial episode **(14)** _____ months ago
- Prescribed **(15)** _____ for 5 days
- Alopurinol
- Recurrence of gout just **(16)** _____
- Gout originates at bottom of **(17)** _____
- on the **(18)** _____ foot
- Swelling radiates across toes towards **(19)** _____ of foot
- Hot and inflammed
- No swelling in **(20)**_____
- Responded well to medication
- Swelling settled within **(21)** _____
- Ongoing **(22)** _____ in toe
- Suspected **(23)** _____
- X-ray clear

Treatment

- Patient has some concerns
- Patient agrees medication effective
- Clinical Decision: **(24)** _____ with current medications

That is the end of Part A.

E2 LANGUAGE

Mock test

LISTENING PART B – QUESTION PAPER

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:
CANDIDATE SIGNATURE _____

Passport Photo

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper**.

DO NOT remove the OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part B & Part C: mark your answers on the **Question Paper** by filling in the circle using a 2B pencil.

Example:

Ⓐ
Ⓑ
Ⓒ

BLANK

Occupational English Test

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A, B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

---

25. You hear a nurse in the emergency department discussing the care of a patient with a doctor.

What do the speakers agree to do?

Ⓐ Put the patient's arm in a sling.

Ⓑ Wait for the patient to calm down.

Ⓒ Treat the patient's pain to relax the arm.

26. You hear a trainee doctor discussing a patient diagnosis with a tutor.

What is the diagnosis?

Ⓐ Coeliac disease

Ⓑ Constipation

Ⓒ Bilateral lower abdominal pain

27. You hear a hospital nurse briefing a colleague about a patient recovering from elective surgery.

What does he want his colleague to do?

Ⓐ review post-op information with the patient

Ⓑ provide pain relief for the patient

Ⓒ get the patient a medical certificate

28. You hear two hospital managers talking about an information session for people who want to do voluntary work.

What problem do the managers discuss?

Ⓐ How difficult it is for participants to get to the event.

Ⓑ Technical issues with the event management systems.

Ⓒ Finding good quality food and drinks for the event.

29. You hear a pharmacist talking to a doctor about a patient's medication.

What is the pharmacist doing?

Ⓐ reporting side effects

Ⓑ checking the dosage

Ⓒ recommending an alternative

30. You hear a doctor advising a patient about a change in medication.

What condition is the patient being treated for?

Ⓐ hay fever.

Ⓑ drowsiness.

Ⓒ hives

That is the end of Part B.

E2 LANGUAGE

Mock test

LISTENING PART C – QUESTION PAPER

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:
CANDIDATE SIGNATURE _____

Passport Photo

INSTRUCTIONS TO CANDIDATES:

DO NOT open this question paper until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, you will have two minutes to check your answers.

At the end of the test, hand in this **Question Paper**.

DO NOT remove the OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Part C: mark your answers on the **Question Paper** by filling in the circle using a 2B pencil.

Example:

Ⓐ
Ⓑ
Ⓒ

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BLANK

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear an interview with Dr Cristine Ericson, who's talking about her research supporting non-fasting lipid blood tests for cholesterol.

You now have 90 seconds to read **questions 31-36**.

31. What is Dr Cristine Ericson's opinion about fasting before blood tests?

- Ⓐ It is not supported by scientific evidence.
- Ⓑ It is superior to non-fasting blood tests.
- Ⓒ It is supported by convention not data.

32. What does Dr Cristine Ericson say about the research on non-fasting blood tests?

- Ⓐ The majority comes from Copenhagen.
- Ⓑ More studies should focus on children.
- Ⓒ It includes data from multiple countries.

33. What is Dr Cristine Ericson's opinion on the difference between the results of fasting and non-fasting tests?

- Ⓐ The small differences are unimportant.
- Ⓑ There is no difference in the results.
- Ⓒ There is a small but significant difference.

34. Dr Cristine Ericson states that everyone in Copenhagen was happy with the introduction of non-fasting blood tests.

What evidence does she provide to support this claim?

Ⓐ Data showing an increase in test attendance.

Ⓑ Increased public demand for non-fasting tests.

Ⓒ Positive stories from other medical professionals.

35. According to Dr Cristine Ericson, why are non-fasting blood tests popular with the public?

Ⓐ Because of the strong research evidence in the report.

Ⓑ Because of positive publicity in the newspaper and on TV.

Ⓒ Because they are more convenient for busy lifestyles.

36. Which of the following statements best summarises Dr Cristine Ericson's position on the use of fasting or non-fasting blood tests?

Ⓐ In rare cases fasting before a blood test is necessary.

Ⓑ Fasting before a blood test should no longer be recommended.

Ⓒ New research is needed on the use of fasting and non-fasting tests.

Now look at extract two.

Extract 2: Questions 37-42

You hear a presentation in which a researcher called Dr Milan Patrecivich is talking about the relationship between new technology and medicine in the future.

You now have 90 seconds to read **questions 37-42**.

37. Dr Patrecivich suggests that in the future medical technology will

- Ⓐ continue to replace human doctors.
- Ⓑ give doctors freedom to express core skills.
- Ⓒ become more accurate and reliable than doctors.

38. According to Dr Patrecivich, how will wearable sensors and smartphones help GPs?

- Ⓐ They will allow doctors mobility to visit patients in remote locations.
- Ⓑ They will allow doctors to check if patients are following their advice.
- Ⓒ They will allow doctors to monitor their patients with real-time data.

39. Dr Patrecivich believes that

- Ⓐ radiologists currently spend too much time checking through images.
- Ⓑ technology like IBM's Medical Sieve isn't yet capable of doing a radiologist's job.
- Ⓒ radiography algorithms should be designed by professional radiologists.

40. According to the talk, both sports medicine and oncology

Ⓐ will use technology to make their processes faster and cheaper.

Ⓑ will use technology to filter data and individualise therapy.

Ⓒ will use technology like Xbox to treat patients from a distance.

41. What does Dr Patrecivich suggest about the effect of technology on cancer treatment?

Ⓐ Artificial intelligence technology will eventually cure cancer.

Ⓑ Oncologists will keep patients informed using social media.

Ⓒ Technology will have a powerful and positive effect on oncology.

42. What is Dr Patrecivich's conclusion about the future of medicine and technology?

Ⓐ Medical technology will give doctors more time and better data to care for patients.

Ⓑ Physicians will have amazing opportunities to develop new medical technologies.

Ⓒ Automated and robotic systems will gradually replace humans in medical practice.

That is the end of Part C.

You now have two minutes to check your answers.

THAT IS THE END OF THE LISTENING TEST

BLANK

E2 Language Listening Part A.2

Extract 1

You will hear part of a consultation between a psychologist and a patient called Mr Barry. For questions **1-12**, complete the notes with a word or short phrase.

Patient:

- Mr Barry

Background:

- Continued anxiety and **(1)**.....
  Started medication **(2)**..... ago

Medication:

- citalopram / chlordiazepoxide
- chlordiazepoxide **(3)**..... Mg **(4)**..... daily
- non-compliant
- feels meds don't **(5)**..... him sufficiently
- patient trying to **(6)**..... more
- citalopram 20mg
- side effects: ongoing **(7)**..... improving
- stress related?
- possible **(8)**.....
- takes citalopram **(9)**.....

Symptoms:

- inability to leave house
- palpitations / SOB / intense fear
- feels **(10)**..... when outside
- patient normalizing
- went to shops for the **(11)**.....
- since beginning **(12)**.....

Extract 2

You will hear part of a consultation between a GP and a patient called Mr Martin. For questions **1-12**, complete the notes with a word or short phrase.

Patient:

- Mr Martin

Background/Symptoms:

- Stomach pain
- Condition present **(1)**..... for years but worse recently
- Located across **(2)**..... of abdomen radiating towards **(3)**.....
- Describes pain as not severe
- Like an (**4)**.....
- Pain moves side to side
- Patient suspects **(5)**..... problems
- Feels **(6)**..... as well as pain
- Ongoing problem
- 4 yrs ago diagnosed suspected **(7)**.....
- Bowel movements variable
- Ongoing problems with **(8)**..... but recently stool **(9)**.....
- Drinks plenty of water
- No diarrhea
- No presence of **(10)**..... in stool
- No black motions

Medication:

- Previously prescribed Colofac
- Monitors diet rather than take **(11)**.....
- Takes OTC **(12)**..... capsules which provide relief

E2 Language Listening Part B.2

1. You hear a doctor and a trainee discussing the application of a plaster cast.

What does the trainee need to tell the patient about the cast?

- ☐ The appropriate length of time to let the cast dry properly
- ☐ The temperature of the cast as it begins to harden.
- ☐ The reduction in the fracture after the cast has been applied.

2. You hear a manager explaining new data management processes to clinical staff.

What should staff do with their feedback reports?

- ☐ File them on paper and digitally
- ☐ Copy them onto the new servers
- ☐ Fill them out digitally to save time

3. You hear a presentation about the introduction of a new type of wound dressing.

What is the main advantage of the new wound dressing?

- ☐ They are ideal for oozing wounds due to high absorbency.
- ☐ They are made from naturally bacteria resistant seaweed.
- ☐ They are somewhat less costly than traditional cloth bandages.

4. You hear two hospital managers discussing completion rates for an online course.

How will management inform staff who do not attend the meeting about the deadline?

- ☐ Face to face
- ☐ By email
- ☐ Over the phone

5. You hear two colleagues discussing an online training course.

What advice does the female nurse give her colleague?

- ☐ request IT help to reset so he can complete the course.

- [ ] wait until the weekend and complete the course.
- [ ] advise a manager that he has completed the course.

6. You hear an educator describing methods for creating medical abbreviations to nursing trainees.

Why does the speaker think it is important for trainees to understand the different methods of abbreviating?

- [ ] So they can create new abbreviations using the first letter rule.
- [ ] So they can easily pronounce medical acronyms accurately.
- [ ] So they can more efficiently learn commonly used abbreviations.

E2 Language Listening Part C.2

Extract 1

1. How does Dr Lee describe the movement from traditional care systems to non-visit care?

   - ☐ Large pre-existing medical providers will probably dominate the market.
   - ☐ There will be a mixture of reactions from existing and new organisations.
   - ☐ Flexible new organisations will have an advantage over existing ones.

2. According to Dr Lee a non-visit care culture

   - ☐ views face to face consultations as a final resort in treatment.
   - ☐ will develop more rapidly in new medical start-ups.
   - ☐ requires a blend of responses from medical organisations.

3. Dr Lee believes that current technology is

   - ☐ closely matched to users medical needs.
   - ☐ changing expectations for medical services.
   - ☐ not capable of supporting non-visit care yet.

4. Dr Lee suggests that in person visits

   - ☐ ideally occur earlier in the care process.
   - ☐ have a negative impact on many patients.
   - ☐ should be easy for patients to manage.

5. Why does Dr Lee mention specimen collection services?

   - ☐ To highlight the expense of creating offices and labs.
   - ☐ To describe how these services could be provided more efficiently.
   - ☐ To suggest they could be delivered more nimbly with technology.

6. What advice does Dr Lee give organisations wanting to move to non-visit care models?

   - ☐ Wait for more precise technology before transitioning to remote care.
   - ☐ Re-evaluate the safety and quality of non-visit care before making the change.
   - ☐ Find transition opportunities by close analysis of each instance of in-person care.

E2 Language Listening Part C.2

Extract 2

1. The research suggests that in the USA

- supervised injection centres are fundamentally different.
- supervised injection centres are being successfully integrated.
- supervised injection centres have the same impact as elsewhere.

2. According to Dr Davidson the secret facility was
   - based on international evidence and good practice.
   - waiting for legal advocacy in the United States.
   - experiencing the death of too many patients.

3. According to Dr Davidson what is the outcome of having staff and drug users in the same room?
   - Patient check in and registration is more efficient.
   - Overdose prevention is the major benefit.
   - Staff can address many potential health issues.

4. What does Davidson suggest users of the centre most appreciate?
   - the clean and hygienic surfaces
   - the private and relaxed space
   - they can't get arrested there

5. Some users of the facility suggested that the facility
   - needs a far more open space.

- can't combine with other services since it is secret.
- would be better as a treatment agency.

6. What concern did staff raise about the running of the facility?

- That patients might disclose information about the centre
- Necessary exclusion of psychologically ill patients
- The small number of patients they could help

E2 Language Listening Part A.3

Extract 1

You will hear part of a consultation between a GP and a patient called Mrs Brownstone. For questions **1-12**, complete the notes with a word or short phrase.

Patient:

- Mrs Brownstone

Background/Symptoms:

- Patient requesting urinalysis
- Recently **(1)**....., burning sensation
- Symptoms present approximately **(2)**..... weeks
- Patient suggests symptoms related to **(3)**.....
- Describes pain as not severe
- Recurring condition
- Treated for similar symptoms **(4)**..... in last **(5)**.....
- Symptoms come and go
- Patient forgets to drink water
- Nocturia - **(6)**..... nightly
- Pain, urgency, **(7)**..... greater at night
- Antibiotics effective in the past
- **(8)**..... urinary incontinence
- No impact from coughing, sneezing
- Successful colposuspension: aged **(9)**.....

Medication:

- Known allergies: **(10)**.....
- Amoxil
- **(11)**.....
- Nitrofurantoine

Treatment Plan:

- Short course nitrofurantoine
- Review appointment in **(12)**.....

E2 Language Listening Part A.3

Extract 2

You will hear part of a consultation between a doctor and a patient called Mrs Chambers. For questions **1-12**, complete the notes with a word or short phrase.

Patient:

- Mrs Chambers

Background/Symptoms:

- Presented after heavy bleeding approximately **(1)**..... ago
- 36 weeks pregnant
- Previous episode at **(2)**..... lasted approx. two hours
- Estimated bleed 1/2 cup
- Current episode **(3)**..... hours
- Ongoing **(4)**..... and passing clots
- Estimated bleed 1 cup
- Tightness / cramps
- Severe pain in **(5)**..... concurrent with bleeding
- Symptoms come and go - "like period pain"
- Pregnancy normal to date
- Previous scan found **(6)**.....
- No further scans scheduled
- 12-week scan showed baby was **(7)**..... but fine
- Antenatal clinic visit normal
- Low **(8)**.....
- Number of previous pregnancies: **(9)**.....
- Boys, **(10)**....., healthy
- Normal delivery
- 4 four years ago-abdominal myomectomy
- Suffered from **(11)**....., body aches, fever
- Emergency **(12)**..... found large fibroids

E2 language Listening Part B.3

1. You hear a doctor and a nurse reviewing a coma patient.

What is the Doctor checking for?

- ☐ The symptoms the patient is exhibiting
- ☐ The severity of the patient's coma
- ☐ The range of mobility of the patient

2. You hear a consultant emergency doctor talking to a patient following an accident.

What is she doing?

- ☐ explaining why she is removing the neck brace
- ☐ clarifying the need for multiple scans and surgery
- ☐ outlining the full extent of the patient's injuries

3. You hear a clinical researcher introducing a professional development workshop.

What is the aim of the workshop?

- ☐ To give participants strategies to gain greater trust from the public.
- ☐ To introduce participants to the basics of mathematical statistics.
- ☐ To lead participants to a deeper critical understanding of research

4. You hear an emergency ward doctor directing her team while treating an accident victim.

What is the doctor most concerned about?

- ☐ The severe injury to the patient's left leg.
- ☐ The possibility of other less apparent injuries.
- ☐ The limited circulation of blood around the injury.

5. You hear a doctor speaking to a woman who has presented with heart palpitations.

What treatment does the patient require?

- ☐ intravenous medication
- ☐ a neck massage
- ☐ training in breath control

6. You hear an emergency paramedic talking to an accident and emergency doctor about a patient who has fallen from his motorcycle.

What is the paramedic doing?

- ☐ confirming what happened to the man in the accident
- ☐ summarising the patient's injuries and treatment
- ☐ suggesting appropriate pain medication to administer

E2 Language Part C.3 Extract 1

1. What is the stated purpose of the talk?

   - [ ] To evaluate if the audience are feeling burnout
   - [ ] To inform and stimulate discussion about burnout
   - [ ] To describe new research on treatment of burnout

2. What did the research from the Mayo Clinic show?

   - [ ] In 2011 more than half of US doctors claimed to have burnt out.
   - [ ] The large number of doctors with burnout is continuing to rise.
   - [ ] Typically one aspect of a doctors work makes them burn out.

3. What is the Anna Capstone's attitude to the problem of burnout?

   - [ ] Most doctors can handle burnout.
   - [ ] Doctors with burnout should retire.
   - [ ] Burnout endangers patients and doctors.

4. How does Anna Capstone think the problem of burnout should be addressed?

   - [ ] Hospitals should fund wellness programs for burnt out staff.
   - [ ] Multiple strategies are needed to keep doctors functioning and healthy.
   - [ ] By discouraging doctors from losing empathy and making mistakes.

5. How does Anna Capstone evaluate the role of administrative technology and regulations?

   - ☐ As a cheap and reliable way of collecting payment.
   - ☐ As confusing and stressful for doctors.
   - ☐ As something few doctors have really complained about.

6. Anna Capston argues that protecting the well-being of highly trained medical staff

   - ☐ is an ethical issue.
   - ☐ isn't part of workplace safety.
   - ☐ will require too many resources.

E2 Language Part C.3 Extract 2

1. Dr MacGregor believes that

   - ☐ most people will find the healthiest diet quite surprising.
   - ☐ intake of meat, eggs, and dairy is an essential part of a balanced diet.
   - ☐ the public is quite well informed about the ideal foods for a healthy diet.

2. What is Dr MacGregor's attitude to dieting?

   - ☐ More people should go on diets.
   - ☐ Dieting is a lifestyle choice.
   - ☐ Diets are generally ineffective.

3. Which industry first "used science against itself"?

   - ☐ The meat industry.
   - ☐ The egg industry.
   - ☐ The tobacco industry.

4. According to Dr MacGregor, in the mid 20th century

   - ☐ the public respected and trusted science to reveal the truth.
   - ☐ smokers clearly understood the risks associated with tobacco use.
   - ☐ the US Surgeon General first reported on the cancer risk from smoking.

5. According to Dr MacGregor the tobacco industry

   - [ ] gathered a mass of evidence on the dangers of smoking.
   - [ ] used its influence to slow down government responses.
   - [ ] has finally recognised that smoking causes lung cancer.

6. The article suggests that the tobacco and food industries share which tactic?

   - [ ] Funding their own research to confuse people.
   - [ ] Using the motto "doubt is our product".
   - [ ] Denying evidence that their products are bad.

7. What do the healthiest and longest living communities in the world have in common?

   - [ ] they eat a lot of legumes.
   - [ ] they are religious communities.
   - [ ] they have a 97% plant-based diet.

Extract 1: Questions 1-12

You hear an obstetrician talking to a patient called Melissa Gordon. For **questions 1-12**, complete the notes with a word or short phrase.

You now have 30 seconds to look at the notes.

Patient

Melissa Gordon

- works as a **(1)**

Medical history

- has occasional **(2)**
- is allergic to **(3)**
- has a **(4)** _____ diet
- non-smoker
- this will be her second child
- needed **(5)** _____ treatment before first pregnancy
- first baby presented as **(6)** _____
  - **(7)** _____ required during intervention
- after giving birth, had problems with **(8)**
  - helped by midwife

Baby's father

- family history of **(9)** _____
- child from previous marriage has **(10)** _____

Points raised

- not keen on amniocentesis
- enquired about the possibility of **(11)** _____ testing
- provided her with a leaflet on preparing **(12)** _____ for new baby

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Extract 2: Questions 13-24

You hear a GP talking to a new patient called Mike Royce. For **questions 13-24**, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes.

Patient Mike Royce

New patient transferring from another practice

Description of initial symptoms

- severe left knee pain in **(13)** _____ area
- worsened after an accident at work
- developed **(14)** _____ on back of knee

(described as trigger points.)

Impact on daily life

- unable to **(15)** _____ while working

(house painter)

- problems climbing ladders

Initial treatment

- exercise programme including
  - stretching exercises
  - rest
- **(16)** _____ for pain

Developments in condition

- GP suspected **(17)** _____
- prescribed hospital-based rehabilitation
- temporary improvement noted

Scanned by CamScanner

Current condition

- muscular problem diagnosed by **(18)** _____
  - was performing treatment on **(19)** _____
- experiencing insomnia and **(20)** _____
- suspects **(21)** _____ (own research)
- has recorded experiences in **(22)** _____
- beginning to experience pain in both **(23)** _____

Suggested course of action

- recommend referral to **(24)** _____

That is the end of Part A. Now look at Part B.

Scanned by CamScanner

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

25. You hear a dietitian talking to a patient.

What is she doing?

(A) correcting the patient's misconception about obesity

(B) describing the link between obesity and other diseases

(C) stressing the need for a positive strategy aimed at weight loss

26. You hear members of a hospital committee discussing problems in the X-ray department.

The problems are due to a delay in

(A) buying a replacement machine.

(B) getting approval for a repair to a machine.

(C) identifying a problem with a particular machine.

27. You hear a senior nurse giving feedback to a trainee after a training exercise.

The trainee accepts that he failed to

(A) locate the CPR board quickly enough.

(B) deal with the CPR board on his own.

(C) install the CPR board correctly.

Scanned by CamScanner

28. You hear a trainee nurse asking his senior colleague about the use of anti-embolism socks (AES) for a patient.

The patient isn't wearing the socks because

(A) she's suffering from arterial disease in her legs.

(B) there is sensory loss in her legs.

(C) her legs are too swollen.

29. You hear a vet talking about her involvement in the management of the practice where she works.

How does she feel about her role?

(A) She accepts that it's become surprisingly complex.

(B) She wishes her boss took more interest in the finances.

(C) She values the greater understanding it gives her of her work.

30. You hear a physiotherapist giving a presentation about a study she's been involved in.

She suggests that her findings are of particular interest because of

(A) the age of the subjects.

(B) the type of disorder involved.

(C) the length of time covered by the study.

That is the end of Part B. Now look at Part C.

Scanned by CamScanner

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear a sports physiotherapist called Chris Maloney giving a presentation in which he describes treating a high jumper with a knee injury.

You now have 90 seconds to read **questions 31-36**.

31. When Chris first met the patient, he found out that

(A) she was considering retirement from her sport.

(B) her state of mind had aggravated the pain in her knee.

(C) she had ignored professional advice previously offered to her.

32. During his assessment of the patient's knee, Chris decided that

(A) her body type wasn't naturally suited to her sport.

(B) the pain she felt was mainly located in one place.

(C) some key muscles weren't strong enough.

33. In the first stage of his treatment, Chris

(A) was careful to explain his methods in detail.

(B) soon discovered what was causing the problem.

(C) used evidence from MRI scans to inform his approach.

34. Why did Chris decide against the practice known as 'taping'?

(A) The patient was reluctant to use it.

(B) It might give a false sense of security.

(C) The treatment was succeeding without it.

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35. In the patient's gym work, Chris's main concern was to ensure that she

(A) tried out a wide range of fitness exercises.

(B) focussed on applying the correct techniques.

(C) was capable of managing her own training regime.

36. Why was the patient's run-up technique changed?

(A) to enable her to gain more speed before take off

(B) to reduce the stress placed on her take-off leg

(C) to reinforce the break from her old mindset

Now look at extract two.

Extract 2: Questions 37-42

You hear a clinical psychiatrist called Dr Anthony Gibbens giving a presentation about the value of individual patients' experiences and 'stories' in medicine.

You now have 90 seconds to read **questions 37-42**.

37. What impressed Dr Gibbens about the case study that was sent to him?

(A) where it was originally published

(B) how controversial its contents were

(C) his colleague's reasons for sending it to him

38. Dr Gibbens has noticed that people who read his books

(A) gain insights into their mental health problems.

(B) see an improvement in personal relationships.

(C) benefit from a subtle change in behaviour.

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39. What disadvantage of doctors using patients' stories does Dr Gibbens identify?

(A) evidence-based research being disregarded

(B) patients being encouraged to self-diagnose

(C) a tendency to jump to conclusions

40. In Dr Gibbens' opinion, why should patients' stories inform medical practice?

(A) They provide an insight not gained from numbers alone.

(B) They prove useful when testing new theories.

(C) They are more accessible than statistics.

41. How does Dr Gibbens feel about randomised medical trials?

(A) He questions the reliability of the method.

(B) He is suspicious of the way data are selected for them.

(C) He is doubtful of their value when used independently.

42. When talking about the use of narratives in medicine in the future, Dr Gibbens reveals

(A) his determination that they should be used to inform research.

(B) his commitment to making them more widely accepted.

(C) his optimism that they will be published more widely.

That is the end of Part C.

You now have two minutes to check your answers.

END OF THE LISTENING TEST

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Extract 1: Questions 1-12

You hear a consultant endocrinologist talking to a patient called Sarah Croft. For **questions 1-12**, complete the notes with a word or short phrase.

You now have 30 seconds to look at the notes.

Patient Sarah Croft

Medical history

- hypertension (recently worsened)
- 3 years of corticosteroid treatment for **(1)** _____

General symptoms

- gradual weight gain, especially in stomach area
- **(2)** _____ on face: embarrassing
- visible **(3)** _____ between the shoulders
- swollen ankles
- excessive and constant **(4)** _____
- backache
- periods are **(5)** _____
- extreme tiredness

Dermatological symptoms

- tendency to **(6)** _____
- wounds slow to heal, **(7)** _____ on thighs
- face appears red in colour, **(8)** _____ area on neck
- recent development of **(9)** _____

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Psychological symptoms

- mildly depressed
- scared by new experience of **(10)** _____
- feels constantly **(11)** _____
- intermittent cognitive difficulties

Recommended tests

- further blood tests
- (12) _____ test possibly

Extract 2: Questions 13-24

You hear an anaesthetist talking to a patient called Mary Wilcox prior to an operation. For **questions 13-24**, complete the notes with a word or short phrase.
You now have thirty seconds to look at the notes.

Patient Mary Wilcox

Current medications

Reason for medication	**Medication**	**Comments**
High blood pressure	Thiazide	both taken this morning with (14) _____
	(13) _____	
Heart attack	(15) _____	taken this morning
	(16) _____	stopped taking this 7 days ago

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Medical history

- went to GP two years ago feeling **(17)** _____ – heart attack subsequently diagnosed
- had two **(18)** _____ inserted

Present condition

- alright with **(19)** _____ and walking on the flat
- has swelling in one ankle following operation for **(20)**
- denies **(21)** _____
- reports some **(22)** _____ at night (responds to medication)

Concerns expressed

- **(23)** _____ following the procedure
- possible damage to crowns (both are **(24)** _____)

That is the end of Part A. Now look at Part B.

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Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

25. You hear two trainee doctors doing an activity at a staff training day.

What does the activity give practice in?

(A) writing case notes

(B) prioritising patients

(C) dealing with consultants

26. You hear a radiographer talking to a patient about her MRI scan.

What is he doing?

(A) clarifying the aim of the procedure

(B) dealing with her particular concerns

(C) explaining how the equipment works

27. You hear two nurses discussing an article in a nursing journal.

What do they agree about it?

(A) It's likely to lead to changes in practice.

(B) It failed to reach any definite conclusion.

(C) It confirms what they were already thinking.

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28. You hear two hospital managers talking about a time management course for staff.

They think that few people have shown interest because

(A) there are so many alternatives on offer.

(B) they feel it's not relevant to them.

(C) it hasn't been publicised enough.

29. You hear an optometrist reporting on some research he's been doing.

The aim of his research was

(A) to develop nanoparticles for transporting drugs all over the body.

(B) to find a way of treating infections caused by contact lenses.

(C) to use contact lenses to administer drugs over time.

30. You hear a consultant talking to a trainee about a patient's eye condition.

What is the consultant doing?

(A) explaining why intervention may not be necessary

(B) suggesting the diagnosis is by no means certain

(C) describing a possible complication

That is the end of Part B. Now look at Part C.

Scanned by CamScanner

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear an interview with a neurosurgeon called Dr Ian Marsh who specialises in the treatment of concussion in sport.

You now have 90 seconds to read **questions 31-36**.

31. Dr Marsh says that one aim of the new guidelines on concussion is

(A) to educate young sportspeople in how to avoid getting it.

(B) to correct some common misunderstandings about it.

(C) to provide a range of specialist advice about it.

32. Dr Marsh makes the point that someone who has suffered a concussion will

(A) be unconscious for varying amounts of time after the event.

(B) need a medical examination before doing any further exercise.

(C) have to take precautions to avoid the risk of symptoms recurring.

33. Dr Marsh says returning to sport too early after a concussion is dangerous because

(A) a subsequent episode can have a cumulative effect.

(B) there is a high risk of fatality in the event of a second one.

(C) the brains of younger people need time to return to normal size.

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34. Dr Marsh suggests that the risk of sustaining a concussion in sports

(A) lies mainly in the choice of sports played.

(B) can be reduced by developing good playing technique.

(C) is greater when sports are played in less formal situations.

35. What is Dr Marsh's view about providing medical support for youth sports events?

(A) Some types of sport are risky enough to justify it.

(B) The organisers should be capable of dealing with any issues.

(C) Certain medical professionals should be encouraged to volunteer.

36. Dr Marsh thinks that developments in college football in the USA

(A) only really address an issue which is particular to that sport.

(B) are only likely to benefit the health of professional sports players.

(C) are a significant step forward in the prevention of concussion in all sports.

Now look at extract two.

Scanned by CamScanner

Extract 2: Questions 37-42

You hear a presentation by a consultant cardiologist called Dr Pamela Skelton, who's talking about a research trial called SPRINT which investigated the effects of setting lower blood-pressure targets.

You now have 90 seconds to read **questions 37-42**.

37. Why was the SPRINT trial stopped before it was due to end?

(A) There were conclusive results earlier than expected.

(B) The high drop-out rate was likely to invalidate the data.

(C) Concerns were raised about possible effects on all participants.

38. A few participants aged over seventy-five left the trial because

(A) there was a negative impact on their daily life.

(B) they failed to take the required doses of medication.

(C) their health deteriorated due to pre-existing conditions.

39. A significant feature of measuring blood pressure in the trial was that

(A) the highest of three readings was recorded.

(B) the patient was alone when it was carried out.

(C) it was done manually by the participant at home.

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40. How did the SPRINT trial differ from the earlier ACCORD study into blood pressure?

(A) SPRINT had fewer participants.

(B) SPRINT involved higher-risk patients.

(C) SPRINT included patients with diabetes.

41. Dr Skelton's main reservation about the SPRINT trial is that

(A) it ignores the wider implications of lowered BP.

(B) its results go against the existing body of evidence.

(C) it was unduly influenced by pharmaceutical companies.

42. What impact does Dr Skelton think the SPRINT trial will have in the future?

(A) It will lead to universally applicable guidelines for BP levels.

(B) Increased attention will be given to the effect of lifestyle on BP.

(C) GPs will adopt a more active approach to lowering BP in the elderly.

That is the end of Part C.

You now have two minutes to check your answers.

END OF THE LISTENING TEST

Scanned by CamScanner

Extract 1: Questions 1-12

You hear a pulmonologist talking to a patient called Robert Miller. For **questions 1-12**, complete the notes with a word or short phrase.

You now have 30 seconds to look at the notes.

Patient Robert Miller

Symptoms

- tiredness
- persistent **(1)** _____ cough
- SOB
- weight loss described as **(2)** _____ in nature.
- fingertips appear **(3)** _____
- nails feel relatively **(4)** _____

Background details

- previously employed as a **(5)** _____ (20 yrs)
- now employed as a **(6)** _____
- no longer able to play golf
- keeps pigeons as a hobby

Medical history

- last year diagnosed with hypertension
- current prescription of **(7)** _____
- **(8)** _____ sounds in chest reported by GP
- father suffered from **(9)** _____

Previous tests

- **(10)** _____ six months ago
- chest x-ray one month ago

Future actions

- **(11)** _____ test
- CT scan
- prescription of **(12)** _____ (possibly)

Extract 2: Questions 13-24

You hear an eye specialist talking to a patient called Jasmine Burton, who has recently undergone eye surgery. For **questions 13-24**, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes.

Patient Jasmine Burton

Patient history

- suffers from **(13)** _____ astigmatism
- also has **(14)** _____ (so surgery under general anaesthetic)
- eye problems may result from a lack of **(15)** _____
- sight problems mean **(16)** _____ isn't an option for her
- reports some slowness to **(17)** _____
- has poor perception of **(18)** _____
- works as a **(19)** _____
  - reports having no issues at work
- eyes checked every few years

Surgery

- **(20)** _____ in right eye first noted three years ago
- February this year - had surgery
- some capsular **(21)** _____ noted post-operatively
- examination showed no sign of a **(22)** _____
  - follow up appointment in 6 months

Presenting with

- reported increase in number of **(23)** _____
- increased sensitivity to **(24)** _____

That is the end of Part A. Now look at Part B.

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For **questions 25-30**, choose the answer (**A**, **B** or **C**) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now look at question 25.

25. You hear a nurse briefing a colleague at the end of her shift.

What does the colleague have to do for the patient tonight?

(A) remove her saline drip

(B) arrange for more tests

(C) monitor her blood pressure

26. You hear part of a hospital management meeting where a concern is being discussed.

What is the committee concerned about?

(A) poor response to recruitment drives

(B) difficulties in retaining suitable staff

(C) relatively high staff absence rates

27. You hear a GP and his practice nurse discussing a vaccination programme.

They agree that the practice should

(A) make sure patients are aware of it.

(B) organise it more effectively than in the past.

(C) prepare to cope with an increasing demand for it.

28. You hear two hospital nurses discussing the assessment of a patient on their ward.

What is the problem?

(A) The patient's documentation has been sent to the wrong place.

(B) Nobody has taken responsibility for assessing the patient.

(C) The duty doctor was unable to locate the patient.

29. You hear the beginning of a training session for dental students.

The trainer is explaining that the session will

(A) focus on aspects of dental hygiene.

(B) expand upon what they studied previously.

(C) introduce them to a completely new technique.

30. You hear two nurses discussing the treatment of a patient with a kidney infection.

What is the female nurse doing?

(A) emphasising the urgency of a procedure

(B) suggesting how to overcome a difficulty

(C) warning him about a possible problem

That is the end of Part B. Now look at Part C.

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For **questions 31-42**, choose the answer (**A, B** or **C**) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

You hear a geriatrician called Dr Clare Cox giving a presentation on the subject of end-of-life care for people with dementia.

You now have 90 seconds to read **questions 31-36**.

31. What problem does Dr Cox identify concerning dementia patients?

(A) to educate young sportspeople in how to avoid getting it.

(B) Their condition can develop in a number of different ways.

(C) to provide a range of specialist advice about it.

32. Why did *Dementia Australia* decide to examine the issue of end-of-life dementia care?

(A) There was a lack of reliable information on it.

(B) The number of stories about poor care made it urgent.

(C) There were enough data on which to base an effective care plan.

33. For Dr Cox, the initial results of the dementia survey reveal that palliative care

(A) was working more effectively than people had thought.

(B) was more widely available than some users imagined.

(C) was viewed negatively by medical professionals.

34. Dr Cox says that lack of knowledge of the law by care professionals

(A) proves that family members should help make pain management decisions.

(B) could be resulting in a surprisingly high number of premature deaths.

(C) may lead to dementia patients experiencing unnecessary distress.

35. Dr Cox thinks that the statistics she quotes on refusing treatment

(A) illustrate a gap in current medical education programmes.

(B) show how patients' wishes are too often misunderstood by carers.

(C) demonstrate the particular difficulties presented by dementia patients.

36. Dr Cox makes the point that end-of-life planning is desirable because

(A) it reduces the complexity of certain care decisions.

(B) it avoids carers having to speculate about a patient's wishes.

(C) it ensures that everyone receives the best possible quality of care.

Now look at extract two.

Extract 2: Questions 37-42

You hear a hospital doctor called Dr Keith Gardiner giving a presentation about some research he's done on the subject of staff-patient communication.

You now have 90 seconds to read **questions 37-42**.

37. Dr Gardiner first became interested in staff-patient communication after

(A) experiencing poor communication as an in-patient.

(B) observing the effects of poor communication on a patient.

(C) analysing patient feedback data on the subject of communication.

38. What point does Dr Gardiner make about a typical admission to hospital?

(A) The information given can overwhelm patients.

(B) Patients often feel unable to communicate effectively.

(C) Filling in detailed paperwork can be stressful for patients.

39. Dr Gardiner uses an example of poor communication to illustrate the point that

(A) patients should be consulted about the desirability of a hospital stay.

(B) specialists need to be informed if there are any mental health issues.

(C) relatives' knowledge of a patient's condition shouldn't be taken for granted.

40. Dr Gardiner explains that a survey conducted among in-patients about communication

(A) measured the difference between their expectations and their actual experience.

(B) asked their opinion about all aspects of the service they received.

(C) included questions on how frequently they visited the hospital.

41. One common complaint arising from Dr Gardner's survey concerned

(A) a lack of privacy for patients receiving sensitive information.

(B) the over-use of unclear medical terminology with patients.

(C) a tendency not to address patients in a respectful way.

42. How does Dr Gardiner feel about the results of the survey?

(A) surprised by one response from patients

(B) reassured by the level of patient care identified

(C) worried that unforeseen problems were highlighted

That is the end of Part C.

You now have two minutes to check your answers.

END OF THE LISTENING TEST